

THE UNION FILE — THE WHOLE PICTURE AND THE ARCHIVE

Cory Shepherd, employee number 388372 · AO3, Switchboard Services, Logan Hospital, Metro South Health · Together Queensland member and endorsed workplace delegate · prepared 3 September 2026

1. THE POSITION AS AT 3 SEPTEMBER 2026

I am employed on a permanent full-time basis, 76 hours per fortnight. My employment commenced on 25 March 2019 and my reinstatement under the agreement of February 2025 preserved continuity of service. Metro South Health described my employment in its own Request for Medical Information of 31 July 2026 as "Permanent Full-time (76 hours per fortnight)".

I have been held out of the workplace since 3 July 2026, and I have received no wages since 13 July 2026 — fifty-two days and four pay days at nil. As a consequence I have given up my accommodation.

On 3 July 2026 Dr Ma completed the Employee Capability Checklist on Metro South's own form. It certifies me fit with restrictions and records, in terms: **"Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below."** The only duty excluded is complaint handling, and even then complaints are to be "logged and redirected, not actioned or resolved". The recommended pattern is six eight-hour shifts per fortnight, which the checklist records as **"the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months"**.

No instrument authorising the exclusion has ever been identified. I first asked for it in the Stage 1 process on 4 August 2026 and have asked repeatedly since. No written decision either returning me to work or refusing to has ever been issued.

2. THE CODING, AND WHAT IT HAS COST

On 2 July 2026 my manager wrote that unless I advised otherwise, shifts I had not worked would be processed as sick leave. On 3 July at 2:49 pm she wrote that she would continue to process them as "Sick leave / Sick leave no pay" because the leave type had not been confirmed. At 3:18 pm the same day I confirmed it in writing: annual leave, not sick leave without pay.

"Sick Leave – No Pay" has been applied to my record every day since. I have asked for it to be corrected on 10 July, 13 July, 28 July, 5 August, 24 August, 31 August and 3 September. The myHR record of 20 July shows leave of the type "Income Protection – No Pay" requested by my line manager on my behalf. My pay advice of 12 August records "NP Sick Leave" for 76.00 hours at \$0.00 gross.

Queensland Health's HR Policy C13 (QH-POL-188), section 6, permits a line manager to submit leave on an employee's behalf **only** where the employee's request for leave is documented in writing and the supporting documents are retained. My written request of 3 July was for annual leave.

The coding has now become the obstacle. On 3 September Payroll advised that my long service leave eligibility date is "only from 04/09/2026". On the service dates recorded in the agreement under which I was reinstated, seven years' continuous service fell due on 25 March 2026. Payroll's date is 163 days later, and unpaid periods are excluded from continuous service. The largest of those is the "Sick Leave – No Pay" applied since 3 July, which I never applied for and contradicted in writing on the day. The employer's own coding has deferred the entitlement now being cited against me, and it moves a day further out for every day it continues.

3. THE REASONS GIVEN FOR NOT PAYING ME — EACH IN THE ARCHIVE

Date	What was said	Tab
2 Jul	Unworked shifts to be processed as sick leave unless otherwise advised	—
15 Jul	Pay "sits with your QSuper Claim Manager"; interim duties refused "in any capacity"; accommodation "remains at the discretion of the Employer"	—
29 Jul	Further medical information required before capacity can be determined	6
30 Jul	The Request for Medical Information is with the delegate for approval	8
31 Aug	Leave applied for one fortnight only, "as you are on reduced roster 0.5 FTE"	17
3 Sep	Long service leave "cannot be backdated"; the ad hoc payment "cannot be processed prior to Wednesday 9th September"	18

Six positions across nine weeks. None of them identifies a power to direct a fit and available employee away from the workplace without pay, and none of them answers the question asked on 4 August.

4. WHAT TOGETHER HAS HELD, AND WHEN

4 October 2025 — my complaint concerning the manager, copied to Heath Moran.

3 November 2025 — endorsed by Heath Moran as a Together workplace delegate, alongside Carolyn Jeffrey and Patricia Conaghan, and working the switchboard roster and consultation issues with him from that date.

14–15 July 2026 — Heath Moran: the matter is "VERY SERIOUS" and is an **urgent referral to Together's industrial team**.

17 July 2026 — Heath Moran: the brief has been delivered in full and the industrial team is engaged and reviewing.

28 July – 3 August 2026 — the instrument-anchored letter to HR and the three Stage 1 emails, each copied to Heath Moran.

11 August 2026 — Emily Petering acknowledges the s 89 letter and engages by telephone.

13–28 August 2026 — the Roberts correspondence, copied to Emily Petering throughout.

21 August 2026 — a two-hour conference with Jacqui Roberts and Terry McQuillan, **Emily Petering present**, across five agenda items. No outcome was reached; the meeting was cut short.

25 August 2026 — Teams invitation to continue that conference, Emily Petering included.

31 August – 3 September 2026 — the leave application, the correction, the ad hoc payment request and the two-part response, each copied to Emily Petering.

2 September 2026 — six asks put to Emily Petering and Heath Moran in writing.

3 September 2026, 2:10 pm — Emily Petering: reviewing the fourteen emails received this week; a reply on the specific questions by the end of the week.

3 September 2026, 2:52 pm — Matthew Beetham, Member Assist: a referral to the Workers' Psychological Support Service, which is described as assisting with work-related injury including "a lack of support from management", and which provides financial counselling.

Seven weeks have passed since the urgent referral to the industrial team of 14 July 2026, and no industrial officer has been identified to me as having carriage of the matter.

5. WHAT REMAINS UNANSWERED

#	The question	Asked	Status
1	The instrument authorising the exclusion	4 Aug 2026, repeatedly since	No answer
2	A written decision returning me to work within the 3 July restrictions, or declining to, identifying the instrument and the officer	4 Aug 2026	Never issued
3	Special leave on full pay under Directive 12/24 cl 6.1 (applied through EB12 cl 9.12)	28 Jul and 5 Aug 2026	Never determined — neither granted nor refused
4	A workplace psychosocial risk assessment under EB12 cl 7.2 and the Psychosocial Code 2022	28 Jul 2026	Not conducted
5	A task-by-task match of the AO3 role description against the Employee Capability Checklist	28 Jul 2026	Not produced
6	Payroll enquiry 4438861	22 Jul 2026	"Pending Investigation"; later shown resolved in myHR, never communicated
7	Ad hoc payment, enquiry 4471091, for the fortnight 3–16 August paid at nil on 26 August	2 Sep 2026	Pending
8	The basis for applying leave at "0.5 FTE" — the last reduced-hours arrangement expired 28 June 2026	31 Aug 2026	No instrument produced
9	The long service leave eligibility calculation, and correction of the coding before it is recalculated	31 Aug and 3 Sep 2026	Leave audit opened 3 Sep; conductor and timeframe unknown
10	Who at Together has carriage of the matter	14 Jul 2026	No industrial officer identified

6. WHAT I AM ASKING THE UNION TO DO

- 1. Carriage.** Confirm in writing who holds this matter and whether an industrial officer is assigned. The referral was made on 14 July. I ask this as an endorsed delegate as well as a member.
- 2. The payroll escalation.** EB12 cl 3.8.2 provides a single payroll point of contact for union officials to escalate payroll errors to, following reasonable steps locally. Those steps are exhausted: on 3 September I wrote to the line manager, to Human Resources, to Payroll as a team and to the Payroll officer by name. I ask that Together make that call.
- 3. The risk assessment.** The psychosocial risk assessment sought under EB12 cl 7.2 on 28 July has never been conducted. That is a workplace matter rather than a personal one, and I ask that Together take it up as its own item for Switchboard Services.

My appeal in the Queensland Industrial Relations Commission is a separate proceeding and nothing in this document forms part of it.

Cory Shepherd · employee number 388372 · 0417 400 227 · coryshepherd1@hotmail.com

7. THE ARCHIVE — EVERY DOCUMENT TOGETHER HAS BEEN SENT OR COPIED ON

Each tab is the original document as sent or received. Nothing has been retyped.

Tab	Date	Between	What it is	Pages
1	4 Oct 2025	Shepherd → Roberts, cc Moran	Complaint concerning the manager, copied to Together	5–9
2	3 Nov 2025	Moran → Shepherd, Jeffrey, Conaghan	■ Endorsement as a Together WORKPLACE DELEGATE; switchboard roster feedback and the Consultation Paper	10–11
3	14–15 Jul 2026	Moran ↔ Shepherd	■ "VERY SERIOUS" — urgent referral to Together's industrial team	12–16
4	17 Jul 2026	Moran → Shepherd	Brief delivered in full; industrial team engaged and still reviewing	17–26
5	28 Jul 2026, 5:39 pm	Shepherd → Injury Management, cc Moran	■ Incorrect application of EB12, the Award and QH policies — 8 instruments; special leave applied for (point 4); psychosocial risk assessment demanded under cl 7.2; task-by-task match of the role description demanded	27–32
6	29 Jul 2026, 4:10 pm	MSH → Shepherd, cc Moran	Reply: further medical information required. Special leave not addressed	33–41
7	30 Jul 2026, 11:25 am	Shepherd → MSH, cc Moran	Roster, pay and leave "remain live in parallel"; ad hoc payment first requested	42–59
8	30 Jul 2026, 2:33 pm	MSH → Shepherd, cc Moran	Holding reply — RFMI with the delegate for approval. Special leave not addressed	60–80
9	3 Aug 2026, 11:38 am	Shepherd → Injury Management, cc HR, Moran	RFMI response	81–84
10	3 Aug 2026, 11:38 am	Shepherd → Injury Management, HR, cc Moran	Request under cl 10.3	85
11	3 Aug 2026, 11:38 am	Shepherd → Taylor, IM, HR, cc Moran	Notice of dispute under cl 1.11 — Stage 1	86
12	4 Aug 2026, 1:35 pm	Taylor → Shepherd, IM, HR, cc Moran	■■ Stage 1 acknowledgement. Concedes the EB12 cl 1.11.2(a) 24-hour timeframe "will not be achieved". States "I am required to process leave on your behalf" for 20 Jul – 2 Aug; records the recreation leave balance at 41.21 hours; and undertakes that absent a response she will apply it "to ensure you receive the available paid leave entitlement for this period". The payslip for that fortnight records NP Sick Leave 76.00 hours, \$0.00	87–90
13	5 Aug 2026, 7:30 am	Shepherd → Taylor, IM, HR	Stage 1 reply — recreation leave not consented to; special leave on full pay sought instead	91–95
14	11 Aug 2026, 4:31 pm	Petering → Shepherd	■ Petering acknowledges the s 89 letter forwarded — first written engagement	96–97
15	18 Aug 2026, 10:47 am	Roberts → Shepherd, cc Petering	"Follow up on Enquiries" re-sent after recall	98–101
16	18–28 Aug 2026	Roberts ↔ Shepherd, cc Petering	Meeting arranged; 24 Aug leave and AVAC requested; 28 Aug AVAC update sought	102–113
17	25 Aug 2026, 3:08 pm	Roberts → Shepherd, McQuillan, Petering	Teams invitation continuing the 21 August conference	114–116
18	31 Aug 2026	Shepherd ↔ Taylor, cc Roberts, Petering	■ The formal application (LSL from 13 Jul, annual leave on exhaustion, AVAC today, under protest); 3:34 pm one fortnight at "0.5 FTE", LSL blocked; 4:06 pm the four-point correction	117–123
19	2–3 Sep 2026	Shepherd ↔ Taylor ↔ Payroll, cc Roberts, Petering	■ Ad hoc payment requested 2 Sep; process 24926366; 3 Sep Payroll "LSL eligibility only from 04/09/2026", leave audit opened; 10:27 no backdating, no ad hoc before 9 Sep	124–138
20	3 Sep 2026, 1:30 pm	Shepherd → Roberts, Taylor, Payroll, cc Petering	■ The two-part response: the coding since 3 July, and payment under HR Policy C13 §§2, 6, 8 and 9; long service leave from 13 July; special leave under Directive 12/24	139–158
21	3 Sep 2026	Together → Shepherd	■ Petering 2:10 pm: reviewing 14 emails, reply by end of week. Beetham 2:52 pm: referral to the Workers' Psychological Support Service	159–165
22	31 Aug 2026	Prepared for Together	The two-page brief: the position in five lines, and every written request for payment	166–167

Total: 163 pages of original correspondence across 22 tabs, 4 October 2025 to 3 September 2026.

Formal Request for Review: Performance Management Process and Handling of Confidential Disclosure

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Cory Shepherd

cory.shepherd@health.qld.gov.au

Sent: Saturday 4 October 2025 at 12:43

To: Ms. Jacquie Roberts, A/Director, HR Business
Partnering

From: Cory Shepherd

Date: October 4th, 2025

Dear Ms. Roberts,

I am writing to formally request your urgent professional intervention regarding a performance improvement plan initiated by my Director, Mr. Scott Hughes, on September 10, 2025. This action was taken without due consideration for a serious personal crisis I am navigating and appears to contravene Queensland Health's policies regarding supportive management and Domestic and Family Violence (DFV) leave.

I am committed to a constructive resolution and trust that your impartial review will facilitate a fair and appropriate outcome. My union representative from Together Heath Moran been copied into the correspondence

Background and Summary of Concerns

The current performance management process stems from a series of procedural and policy failings by management following my confidential disclosure of a personal crisis.

1. Improper Handling of DFV Leave

Request: On March 12, 2025, I confidentially disclosed to Mr. Hughes that I was involved in a complex and traumatic domestic violence situation (involving misrepresentation and disputes over property during separation), as substantiated by court documentation **(Attachment A)**. In accordance with Public Service Commission (PSC) Directive 03/20, I requested access to Domestic and Family Violence Leave **(Attachment B)**. On March 28, 2025, management advised me that as a respondent in the matter, I could be "deemed as the perpetrator" and might be ineligible for leave **(Attachment C)**. This advice is factually incorrect, contravenes the directive, and disregards formal advice I received from the Queensland Police Service, which recommended I seek a protection order for my own safety **(Attachment D)**.

2. Procedural Unfairness and

Misrepresentation: The management approach implemented within the switchboard department has involved punitive performance monitoring, which has proven unproductive given my disclosed difficulties and the adversarial nature of the disclosure. On August 27, 2025, I proactively offered to provide Mr. Hughes' office with relevant documentation to ensure a comprehensive and accurate understanding of my circumstances prior to a scheduled meeting. Immediately following this offer, the meeting was cancelled by his office (**Attachment E**). Subsequently, in his letter initiating the performance plan on September 10, 2025, Mr. Hughes materially misrepresented these events by asserting that my actions were the cause of meetings not proceeding. Ms. Taylor has similarly faced a punitive management approach. (**Attachment F**). A performance process cannot be considered fair or conducted in good faith when an employee's attempts to provide clarifying information are first refused, and that refusal is then used as grounds for punitive action.

3. Inappropriate and Unsupportive

Communication: The pattern of unsupportive management has continued. For instance, on September 29, 2025, shortly after I had finished my shift at 15:00, I received a follow-up email from Chloe, which I have not yet responded to, Ms. Taylor emailed *both* my personal and work email addresses to "follow up to see if I understand the on-call process." This communication felt intrusive and inappropriate, given the timing, the use of my personal email for a routine work query, and the ongoing circumstances. or engaged with, adding to the pressure while I am trying to manage my health and work responsibilities.

The consistent pattern of unsupportive management

remains a concern. On September 29, 2025, immediately after my 3:00 PM shift, I received an intrusive and inappropriate follow-up email from Ms. Taylor. The email, sent to both my personal and work addresses, inquired about my understanding of the on-call process. This communication, particularly its timing and the use of my personal email address, has added to my existing pressure as I manage my health and professional obligations. I have not yet responded to this correspondence.

The cumulative stress from my personal circumstances, compounded by this unsupportive management process, has significantly impacted my well-being. The current performance process unfairly penalizes the symptoms of this crisis rather than addressing the root cause with the support mandated by Queensland Health policy.

Proposed Resolution

To resolve this matter constructively, I respectfully request your assistance with the following:

1. **Immediate suspension of the performance improvement process** to allow for a formal and impartial review of the circumstances leading to its initiation.

2. **An independent review** into management's handling of my confidential disclosure and request for DFV Leave, with specific reference to compliance with PSC Directive 03/20.

3. **A formal audit and correction of my leave balance**, which I believe was improperly coded following my request.

4. **Your appointment as an impartial HR intermediary** to help mediate a fair, supportive, and policy-compliant path forward.

Thank you for your time and attention to this urgent matter. I look forward to your guidance while moving forward in a constructive manner.

Sincerely,

Cory Shepherd

Switchboard Roster Feedback - For Delegates

From: Heath Moran heath.moran@together.org.au

To: Carolyn Jeffrey

Carolyn.Jeffrey@health.qld.gov.au, Patricia

Conaghan Patricia.Conaghan@health.qld.gov.au,

Cory Shepherd Cory.Shepherd@health.qld.gov.au

Cc: Billy Colless billy.colless@together.org.au

Sent: Monday 3 November 2025 at 10:57

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is safe.**

Good morning DELEGATES!

Firstly, congratulations to you all on being endorsed to become workplace delegates!

You'll be receiving some invites to attend our delegate catchups as well as our monthly Local Consultative Forum.

In relation to the switchboard roster feedback, I've attached Chloe's feedback in a PDF attached in this email. Please note that while we're in negotiations with Chloe, this is not for wider distribution.

I have also attached the Consultation paper – which Chloe advised on Thursday last week has the amount of shifts each day that Switch can currently allocate.

I'd be eager to hear your feedback, and I'd like to come together this week to discuss your thoughts. I'll be in the hospital on Thursday around midday for a staff orientation so if you'd like to meet up on Thursday to discuss, please let me know what works for you. Once we've discussed it further amongst ourselves, I'll book in a time to meet with Chloe – likely next week (noting I'll be in Melbourne for work, Billy will be able to attend in my absence).

I look forward to hearing back from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

heath.moran@together.org.au

her.org.au



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RE: Request for contact re workplace issues

From: Heath Moran heath.moran@together.org.au

To: Cory Shepherd coryshepherd1@hotmail.com

Sent: Wednesday 15 July at 15:27

Thank you for providing this Cory,

I'll have time to go through these concerns on Friday as there is significant material here I don't want to make any comments until I've had a chance to review them. Once I've reviewed them I'll get back to you with my thoughts.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

[heath.moran@together](mailto:heath.moran@together.org.au)

[her.org.au](http://together.org.au)



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 5:03 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Hi Heath,

Thanks for getting back to me.

I'm handing over my appeal submissions — not to address the appeal itself, but because they give the fullest picture of the workplace issues I raised.

The documents I attach are for you to read over and identify the risks associated in the role. They are not to be shared but the information I provide in the submissions are to assist in identifying the psychosocial risks I believe that are unassessed.

Furthermore, I have no grievances with Ms Taylor personally; I don't know if the same view is shared but I could be wrong.

Happy to talk any of it through once you've read the attachments.

Form 20 is my affidavit and have I described the role and the workplace. I will send a further email of the completed ECC my Dr has completed, and HR has since told me that I may not return.

Regards,
Cory

From: Heath Moran <heath.moran@together.org.au>

Sent: Tuesday, July 14, 2026 4:04:39 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Subject: RE: Request for contact re workplace issues

Hey Cory,

Thank you for reaching out this is very serious.

For your matter I'll be sending an urgent referral to our industrial team for assistance, if you've been put on an ECC, completed it and are still being told you cannot return, I need to make sure an industrial officer from Together looks over this before anything is actioned as I need to make sure you get the best information.

I'd be very eager to discuss your ongoing concerns for the hazards in switchboard as this is a recurring theme raised by members.

I have time to talk on Thursday morning around 9-10:30am and time sporadically tomorrow to call in between site visits, are you available any time tomorrow or Thursday morning.

Looking forward to hearing from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

[heath.moran@together](mailto:heath.moran@together.org.au)

her.org.au



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 10:50 AM

To: Heath Moran <heath.moran@together.org.au>

Subject: Request for contact re workplace issues

Good morning Heath,

I would like to request a time to speak with you by phone regarding some workplace issues that are causing me serious concern.

Firstly, I understand that Chloe Taylor has informed others that a full-time position will be available soon. My understanding is that this role falls within my line of work, and I would appreciate clarification about the position and my status in relation to it.

Secondly, I have been required to undergo an ECC assessment in order to return to my duties, despite having substantially performed the same role over the past 12 months on reduced hours that I have been able to manage. This requirement was not raised when I previously returned to work following an unfair dismissal matter, which arose after I submitted a public interest disclosure in 2024 that was validated and related to concerns involving Chloe Taylor.

More recently, I was directed not to return to work until my doctor completed the ECC, which has now been done. I am nevertheless still being instructed to remain away from the workplace. I am concerned that this may be connected to my current WorkCover appeal, in which I have raised systemic risks to patient safety arising from how the switchboard is being run, and have explained in detail how those risks contributed to my injury and to subsequent reprisal.

I would welcome the opportunity to discuss these matters with you, including the full-time position, the ECC requirements, and my ongoing exclusion from work. I would also like to share the records I hold, the risks I have identified, and why I consider Switchboard to be an unassessed psychosocial

hazard, including the ways in which its current operation affects both patients and employees, together with the adverse action that I believe is presently being applied against me.

Kind regards

Cory

RE: Request for contact re workplace issues

From: Heath Moran heath.moran@together.org.au

To: Cory Shepherd coryshepherd1@hotmail.com

Sent: Friday 17 July at 15:27

Hey Cory,

To keep you updated I'm still reviewing this information and I'm speaking to our industrial team about best steps forward.

Once I have more I'll let you know.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

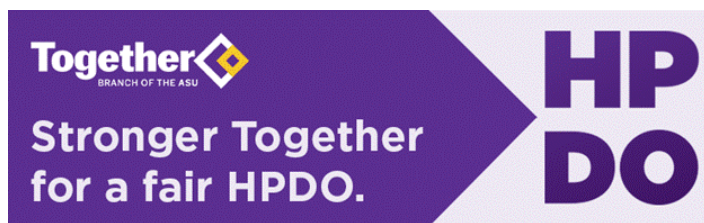
[heath.moran@together](mailto:heath.moran@together.org.au)

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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Thursday, 16 July 2026 4:42 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Good afternoon Heath,

The final piece for your review — the full email chain ending with HR's letter of 15 July 2026 (attached), which sets out their position in writing. I've held off responding pending your review and the industrial team's advice. I will however include you in any further correspondence. To make Friday easier, here is the chain in short, and the issues as I understand them.

What the chain shows

13 July: HR (Michelle Harrison, Injury Management) phoned and then wrote to my QSuper case manager advising the Health Service is "unable to facilitate" my return — before the further medical information they say they need had been requested from my GP.

13 July (me): I asked four things — what was disclosed to the fund and on what authority; that the Request for Medical Information to my GP go out within 3 business days with a copy to me, and the decision timeframe once his response arrives; that my rostered shifts while held out be paid and not taken from my leave; and a response to my standing offer of interim duties.

15 July (their letter): the request to my GP has no timeframe ("unable to commit"); pay is refused on the basis that I "hold a current QSuper Income Protection Claim" (I don't — it is closed, and I've asked the fund for written confirmation); interim duties are refused "in any capacity" pending the un-timetabled GP request; and my situation "remains at the discretion of the Employer" because I don't have an accepted workers' compensation claim, with reference to employees "being managed through Health Management processes."

Issue 1 — the situation is being mischaracterised as a "return to work." I have been continuously employed and working my shifts for the past year, on hours I adjusted within my certified capacity. Throughout that period I kept the fund side correct myself: while the appeal was ongoing I asked QSuper to assess my payslips and adjust my benefit to partial payments reflecting the hours I was actually working, and ultimately my income protection payments stopped at my own request, to prevent overpayment, because I was working. On 3 July at about 2:32pm I provided the Employee Capability Checklist my GP completed certifying me fit to work with identified restrictions — as a serving employee updating my medical information as requested by my employer while finalising my super matters, not as an application for permission to work. I was rostered to work at that time. By about 3:30pm the same day — under an hour — I was held out of the workplace "pending review," with my rostered shifts "paid out of [my] available sick leave." I have been out since, thirteen days now, with no timeframe. Accurately described, I am a working employee directed not to attend by my employer while certified fit — not an absent employee requesting to return. Their letters (including to my super fund) describe me as "a QSuper Claimant request[ing] to return to duties," which is wrong on both counts.

Issue 2 — pay and leave. I am, and at all times since 3 July have been, ready, willing and able to work my rostered shifts; the decision that I not attend is the Health Service's, not mine. No stand-down, suspension or other power has been identified to me, yet the shifts are being debited against my own sick leave — leave that exists to cover incapacity, which on their own file I don't have, and which I have already substantially exhausted through my illness. Their stated basis for refusing pay — that the fund is responsible — rests

on a claim that is closed, and in any event an insurance benefit is not wages.

Issue 3 — the policies and frameworks I believe are being misapplied or ignored. Their letter treats the absence of an accepted workers' compensation claim as leaving everything to "the discretion of the Employer." As I understand it, that is not a complete statement of the position:

(a) Queensland Health's Workplace Rehabilitation Standard (QH-IMP-401-5) provides that rehabilitation, suitable duties and temporary adjustments may be provided to employees without an accepted workers' compensation claim where operationally reasonable, and that managers are responsible for identifying such duties;

(b) Queensland Health's Reasonable Adjustment policy (QH-POL-210) applies to my circumstances;

(c) the Anti-Discrimination Act 1991 (Qld), including the positive duty in force since 1 July 2025, and the Human Rights Act 2019 (Qld) s 58, apply to decisions the Health Service makes about my employment;

(d) the Work Health and Safety Act 2011 (Qld), ss 47–49, requires consultation with me about matters affecting my health and safety. There has been none: the 3 July decision was made and communicated within the hour, and I have not been consulted since — even though their letter now frames the assessment in psychosocial-risk terms.

Issue 4 — "Health Management processes." Their letter says the rehabilitation obligations don't extend to employees "being managed through Health Management processes" — but no one has told me whether I am in such a process, which one, under what policy or statutory instrument, when it commenced, or who the decision-maker is. If this is the Public Sector Act 2022 chapter 3, part 8 pathway, I don't understand how its preconditions

could be met — I was working until directed not to attend, and I hold a current fitness-for-duty certification.

Issue 5 — the request to my GP. There is no timeframe to send it and no timeframe to decide once it's answered, and their letter describes its subject as "the nature of the matters identified by Dr Ma" — which reads wider than the restrictions themselves. I have asked that any request to my GP be copied to me and confined to my capacity, my restrictions, and their operational application.

Issue 6 — the two-year reversal on "psychosocial risk." The psychosocial concerns in this workplace were raised by me, beginning more than two years ago: fatigue and rest-break breaches (including the March 2024 rostering that left me a 7-hour break between shifts), unresolved pay failures, and switchboard workload and communication-flow issues — including in a written grievance on 7 August 2023 that was dismissed the same day it was lodged, and later in the disclosure I mentioned in my first email.

To my knowledge, no psychosocial risk assessment of the switchboard was ever undertaken in response to any of it, and I managed my own exposure over the past year by adjusting my hours within my certified capacity.

The first time the Health Service has applied psychosocial-risk language to this workplace in writing is the 15 July letter — and its subject is not the hazards I raised, but me: whether the "matters identified by Dr Ma" (that is, my medical condition) "present any psychosocial hazards or risks." Two years on, the hazards remain unassessed, and the worker who raised them — and was injured — has become the subject of the assessment. Given you mentioned switchboard hazards are a recurring

theme from members, I'd particularly value the industrial team's view on that sequence.

I had prepared a full response to HR raising all of this, with the policy citations — I'm holding it and won't send anything until you and the industrial team have advised.

One further matter for awareness: while I have been held out, a colleague Caroline has contacted me asking whether I have been dismissed, and I understand the upcoming full-time position I mentioned in my first email has been communicated to casual staff. So, while I am excluded from the workplace with no timeframe, my absence is being noticed by colleagues, and a full-time role in my line of work is being discussed with casuals.

Thanks again for moving quickly on this.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)

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From: Heath Moran <heath.moran@together.org.au>
Sent: Wednesday, July 15, 2026 3:27:25 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Subject: RE: Request for contact re workplace issues

Thank you for providing this Cory,

I'll have time to go through these concerns on Friday as there is significant material here I don't want to make any comments until I've had a chance to review them. Once I've reviewed them I'll get back to you with my thoughts.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

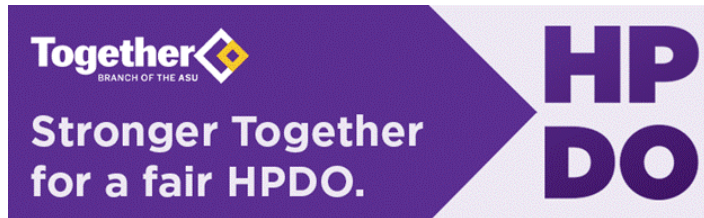
heath.moran@together.org.au

together.org.au



Looking for information? Check the [Member Portal!](#)

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Together recognises and respects the Aboriginal and Torres Strait Islander peoples of the lands upon which we undertake our work to promote and defend the interests and values of members. We acknowledge the Traditional Owners and custodians who have cared for this country for thousands of years.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 5:03 PM

To: Heath Moran <heath.moran@together.org.au>

Subject: Re: Request for contact re workplace issues

Hi Heath,

Thanks for getting back to me.

I'm handing over my appeal submissions — not to address the appeal itself, but because they give the fullest picture of the workplace issues I raised.

The documents I attach are for you to read over and identify the risks associated in the role. They are not to be shared but the information I provide in the submissions are to assist in identifying the psychosocial risks I believe that are unassessed.

Furthermore, I have no grievances with Ms Taylor personally; I don't know if the same view is shared but I could be wrong.

Happy to talk any of it through once you've read the attachments.

Form 20 is my affidavit and have I described the role and the workplace. I will send a further email of the completed ECC my Dr has completed, and HR has since told me that I may not return.

Regards,
Cory

From: Heath Moran <heath.moran@together.org.au>
Sent: Tuesday, July 14, 2026 4:04:39 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Subject: RE: Request for contact re workplace issues

Hey Cory,

Thank you for reaching out this is very serious.

For your matter I'll be sending an urgent referral to our industrial team for assistance, if you've been put on an ECC, completed it and are still being told you cannot return, I need to make sure an industrial officer from Together looks over this before anything is actioned as I need to make sure you get the best information.

I'd be very eager to discuss your ongoing concerns for the hazards in switchboard as this is a recurring theme raised by members.

I have time to talk on Thursday morning around 9-10:30am and time sporadically tomorrow to call in between site visits, are you available any time tomorrow or Thursday morning.

Looking forward to hearing from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

heath.moran@together.org.au

together.org.au



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From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 14 July 2026 10:50 AM

To: Heath Moran <heath.moran@together.org.au>

Subject: Request for contact re workplace issues

Good morning Heath,

I would like to request a time to speak with you by phone regarding some workplace issues that are causing me serious concern.

Firstly, I understand that Chloe Taylor has informed others that a full-time position will be available soon. My understanding is that this role falls within my line of work, and I would appreciate clarification about the position and my status in relation to it.

Secondly, I have been required to undergo an ECC assessment in order to return to my duties, despite having substantially performed the same role over

the past 12 months on reduced hours that I have been able to manage. This requirement was not raised when I previously returned to work following an unfair dismissal matter, which arose after I submitted a public interest disclosure in 2024 that was validated and related to concerns involving Chloe Taylor.

More recently, I was directed not to return to work until my doctor completed the ECC, which has now been done. I am nevertheless still being instructed to remain away from the workplace. I am concerned that this may be connected to my current WorkCover appeal, in which I have raised systemic risks to patient safety arising from how the switchboard is being run, and have explained in detail how those risks contributed to my time off work and to subsequent reprisal.

I would welcome the opportunity to discuss these matters with you, including the full-time position, the ECC requirements, and my ongoing exclusion from work. I would also like to share the records I hold, the risks I have identified, and why I consider Switchboard to be an unassessed psychosocial hazard, including the ways in which its current operation affects both patients and employees, together with the adverse action that I believe is presently being applied against me.

Kind regards

Cory

Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH_InjuryManagement@health.qld.gov.au

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest lyndelle.forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Tuesday 28 July at 17:39

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July

2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General

Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures;

rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.
- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,
Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,
James Zappia james.zappia@art.com.au

Sent: Wednesday 29 July at 16:10

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2744](tel:0735422744)
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @[Solv.com.au](#) email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not

miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_injurymanagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO
NOT click on any links or open attachments unless you
recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard
management correspondence between 2 and 15 July
2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
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- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

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ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.

- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

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- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
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- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl

7.1.5(j) and 7.2.

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- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

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- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.

- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

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ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

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ATT02 EB12 cl 7.1.11 — feedback on WHS matters

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ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.

2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.

3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.

- 4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
- 5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
- 6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
- 7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
- 8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
[0417 400 227](tel:0417400227)
MSH-INJ-5795

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computer network.

**Re: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Thursday 30 July at 11:25

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve

months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022:

hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I

have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health

Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34)

and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of

my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through

Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the

refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au | [ams](#)
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 28 July 2026 5:40 PM
To: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services

(AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02

(EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).

- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.
- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while

working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

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**RE: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: Notes - IPEC (notes@solv.com.au)

notes@solv.com.au

Sent: Thursday 30 July at 14:33

Good afternoon Cory,

Thank you for your email.

Please note that we are currently progressing the Request for Medical Information (RFMI) correspondence for delegate approval and will endeavour to expedite this process as quickly as possible. However, we are unable to guarantee when the approved documentation will be available for release.

The usual process is for the approved RFMI documentation to be sent directly to the employee, who can then arrange an appointment with their treating medical practitioner to have the information completed and returned.

If you have any further questions in the meantime, please do not hesitate to contact us.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) |   

- lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_injurymanagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Thursday, 30 July 2026 11:26 AM
To: LBH Injury Management
<LBH_injurymanagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Re: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

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I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022: hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the

rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34) and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified

restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

4/3-5 University Drive, Meadowbrook Qld 4131

[Metro South Health](#) | 

e. lbh_injurymanagement@health.qld.gov.au | [Chat on Teams](#)

p. [07 3542 2744](tel:0735422744)

a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.

Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 28 July 2026 5:40 PM

To: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>; Lyndelle

Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>; James

Zappia <james.zappia@art.com.au>

Subject: Response to 2-15 July 2026 correspondence -
incorrect application of EB12, Award and QH policies - ECC
3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction

that I not attend "pending review"; • Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma; • your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073); • your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and • leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement
ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015
ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)
ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard
ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)
ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)
ATT13 — QH-POL-231 Special leave (HR Policy C7)
ATT14 — Administration Officer, Switchboard Services (AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated

modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02 (EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).
- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.

- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.

- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).

- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters).

Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.

- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).

- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.

- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while working. Facilitation was then refused and I was directed

off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

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Response — Request for Medical Information, MSH-INJ-5795 [CLM-317073]

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

lbh_injurymanagement@health.qld.gov.au

Cc: LBH_HR lbh_hr@health.qld.gov.au, Heath

Moran heath.moran@together.org.au

Sent: Monday 3 August at 11:38

Dear Human Resources team,

I refer to the email from Injury Management of Friday 31 July 2026 at 11:43am and the two letters of the same date signed by Mr Scott Hughes, Director, Corporate Services. I have directed this response to Human Resources, and copied Injury Management so that it is received within the timeframe the letter set.

Nothing in this response is a personal complaint about any individual, and I ask for no finding about anyone. My concern is with how the process is being run, and with the support and adjustment that ordinarily apply when an employee is certified fit with restrictions. I have copied my union, and I have written separately to the Chief Executive today about the conflict of interest and the handling of my personal information.

I am engaging with this process and will obtain the information sought. My response is attached, with a proposed allocation of the nine questions and my proposal for return to work. In summary:

- Questions 1(d), 4, 5 and 8 I will obtain from my general practitioner; Questions 7, 1(a), 1(d) and 8 from my treating psychiatrist. Please confirm which questions the Health Service presses, and the invoices will be

directed to you under the arrangement in your letter.

- Questions 2, 3, 6 and 9 are not medical questions: they ask my doctor to answer matters which G03 and the Work Health and Safety Act place with the Health Service. I ask that the Health Service answer those itself; the medical part of each can then be put to my practitioners.
- Question 1(a) is answered openly in the attachment: the diagnosis was made on 24 October 2024, with the clinical context to come from my psychiatrist.
- Question 1(c) I consent to, and the answer is in Dr Ma's own file: his understanding of my condition derives from my psychiatrist's correspondence, not my self-report.
- Question 1(b) alone I ask be withdrawn. G03 clause 3 permits medical information "where necessary" to identify or assess adjustments; a causal formulation does neither, and it goes to a matter in issue in WC/2024/227.

I adopt option 1 and will obtain the information myself. I do not provide an authority under option 2.

Three matters are dealt with in the attachment and I flag them here.

First, timing. I met the 2:00pm deadline of 3 July; the checklist certified me fit with restrictions; within the hour I was told I could not attend, and I have been held out for five weeks without an instrument being identified or a date given. Your letter arrived two hours before a consultation with Dr Ma of which the Health Service had notice; I cancelled it rather than waste a cost, and will rebook once the question set is settled. I ask for the extension your letter contemplates, and for a

date by which the Health Service will tell me whether I am returning to work. I also ask what decision is meant by "a decision regarding your ability to perform your role", and under what instrument it would be made — this is the second occasion an adverse position has been foreshadowed without one.

Second, my exclusion. The Health Service's own letter confirms complaints are escalated to the Client Liaison Officer and/or Manager, and the Role Description contains no complaint-handling duty. There is no identified restriction preventing my performance of the role. I ask to be returned to my roster, and that the leave debited since 3 July 2026 be reversed.

Third, delegation and conflict. I previously asked who is making decisions about me and whether any conflict has been identified and managed; I have not been answered. Part 6 of the attachment asks again and proposes the ordinary separation of functions.

I also attach a short notice setting out the arrangements for the medical appointments — which I will rebook within three business days of the Health Service confirming which questions it presses — and the position on the cost of the reports.

I remain willing to return to work on terms which are safe for me and workable for the Health Service, and would be glad to meet with a support person present. My proposal is attached as a separate document, the Proposal for Return to Work, and I ask that it be received together with my separate request of today under clause 10.3.2 of the certified agreement.

Yours sincerely,

Cory Lea Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227 · coryshepherd1@hotmail.com

Request under clause 10.3.2 — change in the way I work — C Shepherd, MSH-INJ-5795

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

lbh_injurymanagement@health.qld.gov.au,

LBH_HR lbh_hr@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au

Sent: Monday 3 August at 11:38

Dear Human Resources team,

I attach a request under clause 10.3.2 of the certified agreement for a change in the way I work. I have made it as a separate document so that there is no question about what it is or when it was received.

The request sets out the change I am asking for and my reasons for it, in the form clause 10.3.3 requires. I have asked that the decision be made by an officer outside Corporate Services, for the reasons at part 4.

I would much prefer this be resolved by discussion and I am available at any time.

Kind regards,

Cory Shepherd

AO3, Switchboard Services, Logan Hospital

[0417 400 227](tel:0417400227)

**Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26
June 2026**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor chloe.taylor3@health.qld.gov.au,

LBH Injury Management

lbh_injurymanagement@health.qld.gov.au,

LBH_HR lbh_hr@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au

Sent: Monday 3 August at 11:38

Dear Chloe,

I attach a notice of dispute under clause 1.11.2(a) of the certified agreement. It is addressed to you as my immediate supervisor because the clause requires that.

I would prefer this resolved at Stage 1 and I am available to discuss it at any time, including today. I have asked that a support person and my union representative be present.

I have raised at part 5 a question about who should conduct the Stage 1 discussion. I make no allegation against anyone and will attend whichever way the Health Service prefers.

Kind regards,

Cory Shepherd

AO3, Switchboard Services, Logan Hospital

[0417 400 227](tel:0417400227)

**RE: Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26
June 2026**

From: Chloe Taylor

Chloe.Taylor3@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au,

LBH_HR LBH_HR@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au

Sent: Tuesday 4 August at 13:35

Hi Cory

I am writing in response to your Stage 1 Dispute dated 3 August 2026, in which you disagree with the decision that has prevented you from attending your rostered shifts since 3 July 2026. Your dispute has been lodged in accordance with clause 1.11.2(a) of the *Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12)*.

I wish to acknowledge that clause 1.11.2(a) of EB12 requires that Stage 1 discussions should be conducted within 24 hours. This letter confirms that your dispute has been received and is currently under review. While every effort has been made to progress this matter promptly, the 24-hour timeframe specified in the clause will not be achieved. Upon completion of the review, you will be contacted as soon as practicable to discuss the outcome of your dispute and any appropriate next steps. You will also be provided with an opportunity to raise any questions and discuss the matter further as required.

Please be advised that I am required to process leave on your behalf for the period 20 July 2026 to 2 August 2026. As at today's date, you have a remaining recreation leave balance of **41.21 hours**. Could you please advise whether you would like this recreation leave balance applied to this period? If so, the remaining hours within the fortnight would be recorded as sick leave without pay. I

acknowledge that your dispute includes concerns regarding your leave entitlements. However, pending resolution of the dispute, it is necessary that your absence continues to be managed through the appropriate leave arrangements.

As the payroll cut-off for PP04 is today, I would appreciate your response as soon as possible. If I do not receive a response before payroll processing occurs, I will apply your remaining recreation leave balance to ensure you receive the available paid leave entitlement for this period.

Employee Assistance

I understand this may be a difficult time for you. Should you wish to seek further support, the Employee Assistance Program, provided through TELUS Health on [1800 604 640](tel:1800604640), is available free of charge for all employees of Metro South Health. This service is a confidential counselling and support service.

Should you wish to access this service, please contact TELUS Health on [1800 604 640](tel:1800604640) or for Aboriginal and Torres Strait Islander support, please contact [1800 816 152](tel:1800816152).

More information about the support TELUS Health can provide can be found out their website <https://app.one.telushealth.com/> or by downloading the free TELUS Health One app. You can log on to both using the following credentials:

Username: mshhs

Password: mshhs

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131



From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Monday, 3 August 2026 11:38 AM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; LBH
Injury Management
<LBH_InjuryManagement@health.qld.gov.au>; LBH_HR
<LBH_HR@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>
Subject: Notice of dispute — clause 1.11 — attendance,
roster, leave and pay from 26 June 2026

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Dear Chloe,

I attach a notice of dispute under clause 1.11.2(a) of the certified agreement. It is addressed to you as my immediate supervisor because the clause requires that.

I would prefer this resolved at Stage 1 and I am available to discuss it at any time, including today. I have asked that a support person and my union representative be present.

I have raised at part 5 a question about who should conduct the Stage 1 discussion. I make no allegation against anyone and will attend whichever way the Health Service prefers.

Kind regards,

Cory Shepherd

AO3, Switchboard Services, Logan Hospital

[0417 400 227](tel:0417400227)

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**Re: Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26
June 2026**

From: Cory Shepherd
coryshepherd1@hotmail.com
To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au,
LBH Injury Management
LBH_InjuryManagement@health.qld.gov.au,
LBH_HR LBH_HR@health.qld.gov.au
Cc: Heath Moran heath.moran@together.org.au,
Emily Petering emily.petering@together.org.au
Sent: Wednesday 5 August at 07:30

Dear Chloe,

Thank you for your response yesterday. My reply is attached. It answers the leave question, sets out my understanding of the position, and asks the questions needed so the correct arrangement can be applied. I have made it a separate document so there is no question about what it is or when it was received.

For payroll purposes, the short position is: I do not consent to my recreation leave being applied, and I have asked that the period be coded as special leave on full pay instead. The detail is in the attachment.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)

From: Chloe Taylor
<Chloe.Taylor3@health.qld.gov.au>
Sent: Tuesday, August 4, 2026 1:35 PM
To: Cory Shepherd <coryshepherd1@hotmail.com>;
LBH Injury Management
<LBH_InjuryManagement@health.qld.gov.au>;
LBH_HR <LBH_HR@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>

Subject: RE: Notice of dispute — clause 1.11 —
attendance, roster, leave and pay from 26 June 2026

Hi Cory

I am writing in response to your Stage 1 Dispute dated 3 August 2026, in which you disagree with the decision that has prevented you from attending your rostered shifts since 3 July 2026. Your dispute has been lodged in accordance with clause 1.11.2(a) of the *Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12)*.

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Please be advised that I am required to process leave on your behalf for the period 20 July 2026 to 2 August 2026. As at today's date, you have a remaining recreation leave balance of **41.21 hours**. Could you please advise whether you would like this recreation leave balance applied to this period? If so, the remaining hours within the fortnight would be recorded as sick leave without pay. I acknowledge that your dispute includes concerns regarding your leave entitlements. However, pending resolution of the dispute, it is necessary that your absence continues to be managed through the appropriate leave arrangements.

As the payroll cut-off for PP04 is today, I would appreciate your response as soon as possible. If I do not receive a response before payroll processing occurs, I will apply your remaining recreation leave balance to ensure you receive the available paid leave entitlement for this period.

Employee Assistance

I understand this may be a difficult time for you. Should you wish to seek further support, the Employee Assistance Program, provided through TELUS Health on [1800 604 640](tel:1800604640), is available free of charge for all employees of Metro South Health. This service is a confidential counselling and support service.

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Username: mshhs

Password: mshhs

Kind regards,
Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 3 August 2026 11:38 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>; LBH_HR

<LBH_HR@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>

Subject: Notice of dispute — clause 1.11 — attendance,
roster, leave and pay from 26 June 2026

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Dear Chloe,

I attach a notice of dispute under clause 1.11.2(a)
of the certified agreement. It is addressed to you as
my immediate supervisor because the clause
requires that.

I would prefer this resolved at Stage 1 and I am
available to discuss it at any time, including today. I
have asked that a support person and my union
representative be present.

I have raised at part 5 a question about who should
conduct the Stage 1 discussion. I make no
allegation against anyone and will attend
whichever way the Health Service prefers.

Kind regards,
Cory Shepherd
AO3, Switchboard Services, Logan Hospital
[0417 400 227](tel:0417400227)

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RE: Attention Ms Noelle Cridland, Chief Executive — Conflict of interest and information handling — C Shepherd, MSH-INJ-5795

From: Emily Petering

Emily.Petering@together.org.au

To: Cory Shepherd coryshepherd1@hotmail.com

Sent: Tuesday 11 August at 16:31

Hi Cory

I'll give you a call once I've looked at the documents you sent through

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, August 11, 2026 4:29 PM

To: Emily Petering <Emily.Petering@together.org.au>;

Heath Moran <heath.moran@together.org.au>

Subject: FW: Attention Ms Noelle Cridland, Chief

Executive — Conflict of interest and information handling — C Shepherd, MSH-INJ-5795

Emily,

Prior to our call it might be valuable to read this email. Noel Cridland has directly acknowledged this email.

Please keep this confidential.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 3 August 2026 11:38 AM

To: noelle.cridland@health.qld.gov.au

Cc: MetroSouthCorro@health.qld.gov.au

Subject: Attention Ms Noelle Cridland, Chief

Executive — Conflict of interest and information handling — C Shepherd, MSH-INJ-5795

Dear Ms Cridland,

I attach a letter concerning the decisions being made about my attendance and my return to duty, and the handling of my personal information.

I would be grateful if your office would bring it to your personal attention: it is a disclosure under section 89 of the Public Sector Act 2022, which the Act directs to the chief executive.

I have addressed it to you because section 89 of the Public Sector Act 2022 makes the chief executive the person to whom a conflict is disclosed, and the only person who may authorise an officer to continue to act in a matter affected by one. I do not raise it with you to escalate it.

I make no allegation against any individual and I ask for no finding to be made about anyone. The letter asks five questions and proposes the ordinary separation of functions. If the answers show the matter has been dealt with, that will resolve it and I will take it no further.

Kind regards,

Cory Shepherd

AO3, Switchboard Services, Logan Hospital

0417 400 227

FW: Follow up on Enquiries

From: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com

Cc: Emily Petering emily.petering@together.org.au,

LBHS Human Resources Team 1

LBH.HRTeam1@health.qld.gov.au, Cory Shepherd

Cory.Shepherd@health.qld.gov.au

Sent: Tuesday 18 August at 10:47

Good Morning Cory

My apologies – I have just noticed I had sent this email to your health account only and did not include your personal, forwarding for your information in case you have not accessed your health email account.

If you could get back to me as soon as possible regarding my meeting suggestion, it would be very much appreciated

Warm Regards

Jacqui

Jacqui Roberts (she / her)

Principal Consultant | Human Resources

Logan and Beaudesert Health Services (LBHS)

Logan Hospital

[Metro South Health](#) | 

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)

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Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

From: Jacqui Roberts

Sent: Thursday, 13 August 2026 4:46 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Cc: LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Emily Petering

<emily.petering@together.org.au>

Subject: Follow up on Enquiries

Good Afternoon Cory

I hope this email finds you well, I wanted to reach out to you in relation to your emails and enquiries you have made over the past couple of weeks and would welcome time to catch up with you to discuss these further to assist in resolving the concerns you have raised.

My availability is a little limited at the moment as I am on leave for the next few days but I do have time on Friday 21 August 2026 and would like to book in a minimum of about 2 hours with you to really discuss and understand your matters further. The times I have available are

- 09:30 to 11:30, or
- 14:30 to 16:30

I propose we can do this via teams and also include Emily Petering too, you will see I have included her in my email.

If you could confirm which of these times would work for you **by close of business on Wednesday 19 August**, that would be really helpful.

Whilst I understand your belief you are fit to return to the workplace; the medical evidence provided to date does not support that. Therefore, this leave would ordinarily be processed as sick leave and if no sick leave balances are available, it will automatically be processed as sick leave no pay. However, to avoid not being paid, with your approval we can apply other leave entitlements, please see below your current balances. If you could confirm what leave you wish to be applied from these balances, and we can action accordingly. You do have sick leave balances, so they should automatically be applied, and this is something we can follow up on for you.

I am aware you have a current open Qsuper claim, and we may need to advise Qsuper of any income you are in receipt of in relation to that claim.

Leave Balance

Request Overview

Leave balances explained

Type	Current Available Balance	Balance as of: 13.08.2025
Recreation Leave	46.62 Hours	
Sick Leave	18.5 Hours	
Reserved Day Off	5.28 Hours	

Looking forward to hearing from you soon. I will be returning to work on Tuesday and happy to respond to any enquiries you may have on my return.

Warm Regards

Jacqui

Jacqui Roberts (she / her)

Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital
[Metro South Health](#) |   

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)
p. (07) 3542 2384 | 0487 313 927

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

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Health for the purposes of operating, protecting,
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computer network.

Re: Follow up on Enquiries

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au

Sent: Friday 28 August at 14:18

Good afternoon Jacqui,

Thank you for your time this week. I have three brief matters to raise.

First, I wish to withdraw the Stage 2 grievance referral. My preference is to work through the outstanding issues directly with you. To be clear, I am withdrawing the referral only. My concerns regarding pay, the leave debited since 3 July 2026, and the adjustments contained in the Employee Capability Checklist dated 3 July 2026 remain unresolved and in dispute. Nevertheless, I am hopeful we can work through these matters constructively.

Second, could you please provide an update on the AVAC following your discussion with Ms Taylor on 24 August?

Third, I attended an appointment with my psychiatrist, Dr Ravikumar Bangalore Krishnaiah, on 27 August 2026. Dr Krishnaiah advised that, despite the authority I provided, Queensland Health has not yet responded to his request for information.

Finally, could someone please contact Dr Krishnaiah's practice so that preparation of the report is not delayed? I would also appreciate being advised of any information being shared with Dr Krishnaiah and the matters on which their opinion is being sought.

And if there are any other updates, I would be glad to have them.

Kind regards,
Cory Shepherd
[0417 400 227](tel:0417400227)
coryshepherd1@hotmail.com

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From: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>
Sent: Monday, August 24, 2026 12:19:27 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: RE: Follow up on Enquiries

Good Afternoon Cory

Thank you for sharing this information with me I will review and come back to you with any questions I may have.

Thank you also for confirming the time to finish our catch up, I will put this in our calendars now and you should receive an invitation very soon.

I additionally acknowledge your requests and I am speaking with Chloe this afternoon in relation to applying your leave.

Warm Regards
Jacqui

Jacqui Roberts (she / her)
Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital
[Metro South Health](#) | 
e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)
p. (07) 3542 2384 | 0487 313 927

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Monday, 24 August 2026 11:13 AM
To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: Re: Follow up on Enquiries

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Good morning,

I apologise for Friday and for the state I was in.

I can confirm I am available on Tuesday, 25 August 2026, between 3:00 pm and 5:00 pm.

For ease of reference, I attach:

- the QSuper letter
- the communication I believe prompted the ECC
- my movement details
- my agreement to the sharing of medical information and correspondence with my treating psychiatrist

In the circumstances, I ask to be advised, in advance, of what information is being shared with my treating psychiatrist, what questions are being asked of them, and what is proposed to be included in any correspondence with them.

In the interim, while Metro South Health seeks clarification regarding my medical information, I respectfully request that the appropriate leave be applied and that an AVAC be submitted on my

behalf to facilitate the processing of my entitlements.

Kind regards,
Cory

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From: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>
Sent: Monday, August 24, 2026 7:44:58 am
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Emily Petering <emily.petering@together.org.au>;
Terry McQuillan <Terry.McQuillan@health.qld.gov.au>
Subject: RE: Follow up on Enquiries

Good Morning Cory

Sorry technology let us down on Friday to finalise our catch up.

I have the following availability for us to conclude our discussions, can you advise which of these times would work for you, Emily has confirmed she is also available to attend at these times too

- Tuesday 25 August 2026 – Between 3pm and 5pm, or
- Friday 28 August 2026 – Between 9am and 11am

If you let me know when you can and I will book it in for us all.

Warm Regards
Jacqui

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Tuesday, 18 August 2026 5:22 PM

To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>

Subject: Re: Follow up on Enquiries

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safe.**

Hey Jacqui,

I can confirm that 14:30 to 16:30 works for me.

Thanks for organising this, and I look forward to catching up with you and Emily on Friday.

Kind regards,

Cory

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From: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>

Sent: Tuesday, August 18, 2026 3:22:54 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Emily Petering <emily.petering@together.org.au>;

LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd

<Cory.Shepherd@health.qld.gov.au>

Subject: RE: Follow up on Enquiries

Good Afternoon Cory

Thank you for coming back to me and also listing all of your points you would like to discuss.

Emily has reached out this afternoon to advise she is no longer available on Friday morning – would you be free on Friday afternoon?

Thank you
Jacqui

Jacqui Roberts (she / her)
Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital
[Metro South Health](#) | 

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)
p. (07) 3542 2384 | 0487 313 927

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 18 August 2026 11:33 AM
To: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>
Cc: Emily Petering <emily.petering@together.org.au>;
LBHS Human Resources Team 1
<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd
<Cory.Shepherd@health.qld.gov.au>
Subject: Re: Follow up on Enquiries

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Dear Jacqui,

Thank you for your email and for including Emily Petering.

I confirm the meeting on Friday, 21 August, from 9:30 am to 11:30 am via Teams, with Emily attending. To ensure the time is used effectively, I expect to discuss the following matters:

- Restoration of my pay, including my request of 28 July 2026 regarding paid special leave.

- Recrediting of all leave debited since 3 July 2026.
- Implementation of the adjustments certified in the Employee Capability Checklist dated 3 July 2026, and my return to work.
- My requests of 3 August 2026, including the flexible working request under the Agreement, for which the decision period ends on 24 August 2026.
- The Stage 2 grievance referral.
-

If there are additional matters you intend to raise, I would appreciate receiving notice of them before Friday.

Medical position – correction

For accuracy, fitness to return to work with adjustments is not my personal view. It is the certification provided by Dr Ma in the Employee Capability Checklist dated 3 July 2026, completed on the Health Service’s own form and at the Health Service’s request.

In addition, a specialist psychiatric report responding to the Health Service’s request for medical information dated 31 July 2026 has been commissioned and will be provided once completed.

•

Leave

To maintain my income in the interim, I do not object to the Health Service processing my available sick leave and, if necessary, annual leave. However, I wish to be clear about the basis on which this occurs:

-
- Any leave processing is being undertaken by the Health Service to ensure continuity of payment and does not represent an election by me.

- My position remains as set out on 3 August 2026: this absence results from the Health Service's decision and should not be debited against my leave balances.
- This arrangement is interim only and subject to the specialist psychiatric report. Once that report is provided, I expect the certified adjustments to be implemented, my return to work to proceed, and my pay to be restored.
- All leave debited since 3 July 2026 should be recredited.
-

Please also review my payslip dated 12 August, which recorded 76 hours as sick leave without pay despite the balances identified in your email. If an AVAC is required to process the leave and correct the payslip, please arrange for this to be submitted promptly and advise if anything further is required from me.

-

QSuper

For accuracy, benefit payments ceased earlier this year following my request to the fund when I returned to work. I have not received any income protection payments since 31 May 2026 and am not currently in receipt of such payments.

The administration of the claim is a matter I am addressing directly with the fund's Claims Manager. Superannuation contributions continue to be reported through payroll in the ordinary course.

-

Thank you for arranging this meeting. I look forward to speaking with you on Friday.

Kind regards,
Cory Lea Shepherd

From: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>

Sent: Tuesday, August 18, 2026 10:47:48 am

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Emily Petering <emily.petering@together.org.au>;

LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Cory Shepherd

<Cory.Shepherd@health.qld.gov.au>

Subject: FW: Follow up on Enquiries

Good Morning Cory

My apologies – I have just noticed I had sent this email to your health account only and did not include your personal, forwarding for your information in case you have not accessed your health email account.

If you could get back to me as soon as possible regarding my meeting suggestion, it would be very much appreciated

Warm Regards

Jacqui

Jacqui Roberts (she / her)

Principal Consultant | Human Resources
Logan and Beaudesert Health Services (LBHS)
Logan Hospital

[Metro South Health](#) | 

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)

p. [\(07\) 3542 2384](tel:(07)35422384) | [0487 313 927](tel:0487313927)

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambah, Quandamooka, Jaggera, Ugarapul, Turrbal and Mununjali peoples – and to Elders, past and present.

From: Jacqui Roberts

Sent: Thursday, 13 August 2026 4:46 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Cc: LBHS Human Resources Team 1

<LBH.HRTeam1@health.qld.gov.au>; Emily Petering

<emily.petering@together.org.au>

Subject: Follow up on Enquiries

Good Afternoon Cory

I hope this email finds you well, I wanted to reach out to you in relation to your emails and enquiries you have made over the past couple of weeks and would welcome time to catch up with you to discuss these further to assist in resolving the concerns you have raised.

My availability is a little limited at the moment as I am on leave for the next few days but I do have time on Friday 21 August 2026 and would like to book in a minimum of about 2 hours with you to really discuss and understand your matters further. The times I have available are

- 09:30 to 11:30, or
- 14:30 to 16:30

I propose we can do this via teams and also include Emily Petering too, you will see I have included her in my email.

If you could confirm which of these times would work for you **by close of business on Wednesday 19 August**, that would be really helpful.

Whilst I understand your belief you are fit to return to the workplace; the medical evidence provided to date does not support that. Therefore, this leave would ordinarily be processed as sick leave and if no sick leave balances are available, it will automatically be processed as sick leave no pay. However, to avoid not being paid, with your approval we can apply other leave entitlements, please see below your current balances. If you could confirm what leave you wish to be applied from these balances, and we can action accordingly. You do have sick leave balances, so they should automatically be applied, and this is something we can follow up on for you.

I am aware you have a current open Qsuper claim, and we may need to advise Qsuper of any income you are in receipt of in relation to that claim.

Leave Balance		Report Overview
Leave balances explained		
Type	Current Available Balance	Balance as of: 13.08.2020
Recreation Leave	46.82 100%	
Sick Leave	15.9 100%	
Restated Day Off	5.28 100%	

Looking forward to hearing from you soon. I will be returning to work on Tuesday and happy to respond to any enquiries you may have on my return.

Warm Regards

Jacqui

Jacqui Roberts (she / her)

Principal Consultant | Human Resources

Logan and Beaudesert Health Services (LBHS)

Logan Hospital

Metro South Health |   

e. jacqui.roberts@health.qld.gov.au | [Chat on Teams](#)

p. (07) 3542 2384 | 0487 313 927

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Health for the purposes of operating, protecting,
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computer network.

Meeting - Cory Shepherd

From: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,

Terry McQuillan

Terry.McQuillan@health.qld.gov.au, Emily Petering

emily.petering@together.org.au

Sent: Tuesday 25 August at 15:08

****Update 25/08/2026 – For Internal information only –
adding in meeting room, not relevant for Emily or Cory****

Good Afternoon Cory

As arranged, here is the link to join the meeting to
continue our discussions from Friday.

Warm Regards

Jacqui

Microsoft Teams meeting

Join:

[https://teams.microsoft.com/meet/411](https://teams.microsoft.com/meet/411575551338689?)

[575551338689?](https://teams.microsoft.com/meet/411575551338689?)

[p=zBTEMPwfHn13HT1TLE](https://teams.microsoft.com/meet/411575551338689?)

Meeting ID: 411 575 551 338 689

Passcode: dF6EL6MP

[Need help?](#) | [System reference](#)

Dial in by phone

[+61 7 3186 0111,,735268758#](tel:+61731860111735268758) Australia, Brisbane

[Find a local number](#)

Phone conference ID: [735 268 758#](tel:735268758)

Join on a video conferencing device

Tenant key: qldhealth@m.webex.com

Video ID: 137 555 063 4

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For organizers: [Meeting options](#) | [Reset dial-in PIN](#)



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**Re: Cory Shepherd (388372) —
application for long service leave and
annual leave, and request for AVAC to be
processed today**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au

Cc: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au,

emily.petering@together.org.au

emily.petering@together.org.au

Sent: Monday 31 August at 16:06

Dear Chloe,

Thank you for actioning this today and for the reference number — I appreciate the quick turnaround.

Four short points so the record is accurate.

1. Leave type. To confirm my application: I did not apply for sick leave — I am not unfit for work, and the Employee Capability Checklist of 3 July 2026 certifies me fit with restrictions. My application was for long service leave and, on its exhaustion, annual leave. Could you please confirm exactly which leave types and how many hours were applied for 17–30 August 2026 under PRN 249 863 66. If any personal or sick leave has been debited, could it please be re-credited and replaced in accordance with my application.

2. Hours. Could the leave please be applied at my substantive hours of 76 per fortnight rather than at 0.5 FTE. The reduced-hours arrangements this year were temporary changes to working conditions: the most recent, approved on 9 June 2026, had an effective period of 25 May 2026 to 28 June 2026 and was not extended.

No variation has been in force since 28 June 2026, and my pay advice of 12 August 2026 itself records my absence at 76.00 hours for the fortnight. If a different arrangement has been recorded for any period after 28 June 2026, could you please let me know what it is and the document it is based on.

3. The period from 13 July. My application was from 13 July 2026, being the date from which I have received no wages. I understand the long service leave question is with Payroll — thank you for raising it promptly. Once it is resolved, could the leave please be applied back to 13 July 2026 as an adjustment, and the "NP Sick Leave" recorded for the earlier fortnights corrected at the same time.

4. Long service leave — what may assist Payroll. My employment commenced on 25 March 2019, and my reinstatement in February 2025 took effect from 20 September 2024 with continuity of service preserved. I therefore reached seven years' continuous service on 25 March 2026 and am entitled to take long service leave under clause 22(c) of the Award. If the system is showing a later continuous service date, it may not have been corrected following my reinstatement — that would explain the error, and Payroll can verify it against my record. I have separately written to Payroll today about my service record and leave balances. Could you also confirm that the payment for 17–30 August will be processed urgently?

For completeness, everything actioned today, and any adjustments, remain on the basis set out at paragraph 4 of my email this morning — under protest and subject to re-credit.

Thanks again, Chloe.

Kind regards,
Cory Shepherd

Employee number 388372

0417 400 227

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Monday, August 31, 2026 3:34:24 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Good afternoon Cory,

Thank you for your email. I have applied your S/L and A/L in lieu of S/L for your absence from the workplace through an AVAC as requested, for the past fortnight 17th August 2026 – 30th August 2026. As you are on reduced roster 0.5 FTE, I have applied the leave as per reduced shifts - PRN 249 863 66

About your long service leave from the 13th July 2026 - the system is currently not allowing me to apply LSL, so I have made enquiries with Payroll to understand why this may be and I will get back you as soon as I have an update.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 31 August 2026 10:59 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au

Subject: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

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you recognise the sender and know the content is
safe.**

Dear Chloe,

I am writing to apply for leave and to ask that the AVAC be processed today. I have copied Jacqui Roberts.

1. MY POSITION

I have not been working since 3 July 2026. I have received no wages since 13 July 2026.

On 3 July 2026 Dr Ma completed the Employee Capability Checklist on Metro South's own form. It certifies me fit with restrictions and records that usual switchboard operational duties remain suitable. I have remained ready and available to work within those restrictions throughout, and I remain so now.

My pay advice of 12 August 2026, for the fortnight 20 July to 2 August 2026, records me as on "NP Sick Leave" for 76 hours with gross pay of \$0.00. I did not apply for sick leave and I am not unfit for work.

2. WHAT I APPLY FOR

In the interim, and so that my entitlements can be processed, I apply as follows:

- Long service leave, on full pay, from 13 July 2026 being the date from which I have received no wages through to the current date and continuing; and
- when my long service leave entitlement is exhausted, annual leave, to commence immediately from that point and to continue until further notice.

To the extent that any absence over that period has already been recorded as unpaid sick leave, I ask that the coding be corrected and replaced with the leave applied for above.

I confirm I am not applying for personal leave, as I am not unfit for work.

3. THE AVAC

I ask that the AVAC be submitted and processed today, and that I be given the reference number once it has been done, together with confirmation of the pay run in which the payment will be made.

I ask this because of the financial position set out below, and because the cut-off for the next available pay run is imminent.

4. THIS APPLICATION IS MADE UNDER PROTEST, AND SUBJECT TO RE-CREDIT

This application is made under protest and without prejudice to my position, and on the following express basis:

- I remain ready, willing and able to work within the restrictions recorded in the Employee Capability Checklist of 3 July 2026, and I have been since 3 July 2026;
- I apply for this leave only because I have received no wages since 13 July 2026 and no payment has been made to me in the interim;

- my concerns regarding pay, the leave debited since 3 July 2026, and the adjustments recorded in the Employee Capability Checklist of 3 July 2026 remain unresolved and in dispute, and nothing in this application resolves, waives or narrows any of them; and
- I ask that the leave taken under this application be re-credited to me in the event that wages are paid, or found payable, for any part of the same period, or in the event that the recording of my absence over that period is later corrected or determined otherwise.

I make this application on the understanding that it does not prevent that outcome, and I reserve my position in full.

5. FINANCIAL HARDSHIP

I want to be plain about why I am asking for this to be done today rather than in the ordinary course.

I have had no income since 13 July 2026. As a consequence I have had to give up my accommodation. I am not raising this to make a point; I am raising it because it is the reason for the urgency, and because I would rather tell you than have it become apparent later.

If there is any mechanism available for an urgent or off-cycle payment in cases of financial hardship, I would be grateful if you could tell me and I will apply for it.

6. IF ANY PART OF THIS CANNOT BE ACTIONED

If any part of this application cannot be actioned today, could you please let me know today which part, and why, and when it will be actioned. A short note is entirely sufficient.

Thank you for your assistance.

Kind regards,
Cory Shepherd
Employee number 388372
0417 400 227
coryshepherd1@hotmail.com

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RE: Cory Shepherd (388372) RE: Please respond to Chloe Taylor regarding long service leave and why it can not be actioned.

From: Chloe Taylor

Chloe.Taylor3@health.qld.gov.au

To: Cory Shepherd coryshepherd1@hotmail.com,
PayrollMetroSouth

PayrollMetroSouth@health.qld.gov.au

Cc: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au, Emily Petering
emily.petering@together.org.au

Sent: Thursday 3 September at 10:27

Hi Cory,

Please see the below email from Payroll.

I understand this may be frustrating, particularly in relation to your Long Service Leave. I have requested further information from Payroll regarding why your eligibility date is showing as 04/09/2026, as I appreciate you would like some clarity around this.

Unfortunately, Long Service Leave cannot be backdated & paid prior to the date you become eligible.

I have also confirmed with Payroll this morning on the phone regarding the ADHOC payment for the 17/08/2026–30/08/2026 pay cycle. Unfortunately, this cannot be processed prior to Wednesday 9th September 2026.

I appreciate this may not be the outcome you were hoping for, and I will continue to follow up where I can, to ensure everything is clarified and processed as soon as possible.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



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From: PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>

Sent: Thursday, 3 September 2026 9:05 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Subject: RE: Cory Shepherd (388372) RE: Please respond to
Chloe Taylor regarding long service leave and why it can
not be actioned.

Hi Chloe,

You aren't able to put in LSL leave as Cory's LSL
eligibility date is only from 04/09/2026.

I can see a log has been created for a leave audit, a
CSO is currently working on it however still in
progress.

Kind Regards,



John Layson

Client Service Officer, Metro South
Payroll Team, Meadowbrook Service
Centre

Payroll Transactional Services, Corporate
Enterprise Solutions, Corporate Services Division,
Queensland Health | Queensland Health

P 3608 5985

E John.Layson@health.qld.gov.au

W health.qld.gov.au

A Level 1, 11-13 University Drive, Meadowbrook

QLD 4131



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past, present and future.

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Wednesday, 2 September 2026 4:16 PM

To: Cory Shepherd <coryshepherd1@hotmail.com>;

PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>

not be actioned.

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safe.**

Dear Chloe,

Thank you for your correspondence confirming that all AVAC submissions lodged on 31 August 2026 have been processed.

I have now successfully reset myHR and regained access to my account. After reviewing the records available to me, I have been unable to locate any enquiry, submission, correspondence, or documentation relating to the AVAC request associated with PRN 249 863 66.

In addition, I contacted Payroll by telephone today at 13:24pm and was advised that there is no information recorded against PRN 249 863 66. Payroll further confirmed that all AVAC submissions lodged on 31 August 2026 had been processed on that date.

I was also advised by Payroll that long service leave or where a leave type cannot be processed, or where a request is rejected, the reason is ordinarily identified during the actioning process. In light of this advice, I would be grateful if you could clarify what information or response you are currently awaiting from Payroll in relation to my long service leave application.

Given that I have been advised on two occasions—on 31 August 2026 and again on 2 September 2026—that an enquiry had been submitted, I would appreciate receiving copies of the following:

- The enquiry, submission, or request that you have confirmed was lodged.

- Any correspondence, records, or communications sent to Payroll regarding my long service leave application.
- Any documentation confirming the processing, status, or outcome of the AVAC request.
- Any correspondence relating to difficulties in processing my long service leave, including any advice received as to why the leave could not be applied.
- Any responses received from Payroll concerning my long service leave entitlement or application.
-
- This matter is of considerable importance to me. The ongoing delay in resolving my leave and pay arrangements has caused significant personal hardship, and I am seeking to establish a clear understanding of the steps that have been taken and any outstanding issues that remain.
-
- To assist in resolving the matter with yourself and payroll as quickly as possible, I would be grateful if all relevant correspondence, enquiries, and records could be provided at your earliest convenience.
-

I appreciate your assistance and look forward to your response.

Kind regards,

Cory

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Wednesday, September 2, 2026 1:00:56 pm

To: PayrollMetroSouth

<payrollmetrosouth@health.qld.gov.au>; Emily Petering

<emily.petering@together.org.au>

Subject: Fw: Cory Shepherd (388372) RE: Please respond to Chloe Taylor regarding long service leave and why it can not be actioned.

Good afternoon,

Please respond to Chloe Taylor and resolve the issue with long service leave.

Kind regards,

Cory Shepherd

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Wednesday, September 2, 2026 12:42:58 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>; Terry McQuillan

<Terry.McQuillan@health.qld.gov.au>

Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Hi Cory,

Thanks for your email.

PRN 249 863 66 has been processed for the fortnight ending 30th August 2026 and will be paid in the normal pay cycle on Wednesday 9th September 2026. I have applied S/L and A/L in lieu of S/L for this pay cycle.

Regarding your Long Service Leave from 13 July 2026, the system is currently not allowing me to apply the leave requested. I have made enquiries with Payroll and am currently awaiting their advice. I will provide an update once I receive further information.

Please see below your current leave balances as of 2nd

September 2026 – Please note that leave balances shown below are prior to AVAC being processed and may therefore change once pay cycle is complete.



If you are having trouble with access, you can call IT Support [1800 198 175](tel:1800198175) and reset your log in details.

Kind regards,
Chloe

Chloe Taylor
Switchboard Manager | Local Telecommunications Representative | Switchboard Services
Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190
E chloe.taylor3@health.qld.gov.au
W www.health.qld.gov.au
A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
T [Call me on Teams](#)



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From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Wednesday, 2 September 2026 11:07 AM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>; emily.petering@together.org.au
Subject: Re: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

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safe.**

Follow up

Dear Chloe,

I am asking for an ad hoc payment to be processed today.

The leave has already been applied and the AVAC processed under PRN 249 863 66, so the entitlement exists, I am asking only that it be paid now rather than held to the next pay run.

I have received no wages since 13 July 2026. That is fifty-one days and three pay runs, and I have already had to give up my accommodation.

I should also mention that I am currently unable to log into MyHR. I cannot submit a leave application, check my balances, or see what has been applied to my record, so I am dependent on you and on Payroll for both the payment and for confirmation of what has been processed. If my access can be restored, I would be grateful and, in the meantime, could the information simply be provided to me by email.

If you cannot raise an ad hoc payment yourself, could you please tell me today who can, and I will make the request to them directly.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

coryshepherd1@hotmail.com

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, August 31, 2026 4:06:26 pm

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: Re: Cory Shepherd (388372) — application for

long service leave and annual leave, and request for AVAC to be processed today

Dear Chloe,

Thank you for actioning this today and for the reference number — I appreciate the quick turnaround.

Four short points so the record is accurate.

1. Leave type. To confirm my application: I did not apply for sick leave — I am not unfit for work, and the Employee Capability Checklist of 3 July 2026 certifies me fit with restrictions. My application was for long service leave and, on its exhaustion, annual leave. Could you please confirm exactly which leave types and how many hours were applied for 17–30 August 2026 under PRN 249 863 66. If any personal or sick leave has been debited, could it please be re-credited and replaced in accordance with my application.

2. Hours. Could the leave please be applied at my substantive hours of 76 per fortnight rather than at 0.5 FTE. The reduced-hours arrangements this year were temporary changes to working conditions: the most recent, approved on 9 June 2026, had an effective period of 25 May 2026 to 28 June 2026 and was not extended.

No variation has been in force since 28 June 2026, and my pay advice of 12 August 2026 itself records my absence at 76.00 hours for the fortnight. If a different arrangement has been recorded for any period after 28 June 2026, could you please let me know what it is and the document it is based on.

3. The period from 13 July. My application was from 13 July 2026, being the date from which I have received no wages. I understand the long service leave question is with Payroll — thank you for raising it promptly. Once it is resolved, could the leave please be applied back to 13 July 2026 as an

adjustment, and the "NP Sick Leave" recorded for the earlier fortnights corrected at the same time.

4. Long service leave — what may assist Payroll.

My employment commenced on 25 March 2019, and my reinstatement in February 2025 took effect from 20 September 2024 with continuity of service preserved. I therefore reached seven years' continuous service on 25 March 2026 and am entitled to take long service leave under clause 22(c) of the Award. If the system is showing a later continuous service date, it may not have been corrected following my reinstatement — that would explain the error, and Payroll can verify it against my record. I have separately written to Payroll today about my service record and leave balances. Could you also confirm that the payment for 17–30 August will be processed urgently?

For completeness, everything actioned today, and any adjustments, remain on the basis set out at paragraph 4 of my email this morning — under protest and subject to re-credit.

Thanks again, Chloe.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Monday, August 31, 2026 3:34:24 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Good afternoon Cory,

Thank you for your email. I have applied your S/L and A/L in lieu of S/L for your absence from the workplace through an AVAC as requested, for the past fortnight 17th August 2026 – 30th August 2026. As you are on reduced roster 0.5 FTE, I have applied the leave as per reduced shifts - PRN 249 863 66

About your long service leave from the 13th July 2026 - the system is currently not allowing me to apply LSL, so I have made enquiries with Payroll to understand why this may be and I will get back you as soon as I have an update.

Kind regards,
Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

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A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, 31 August 2026 10:59 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au

Subject: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

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safe.**

Dear Chloe,

I am writing to apply for leave and to ask that the AVAC be processed today. I have copied Jacqui Roberts.

1. MY POSITION

I have not been working since 3 July 2026. I have received no wages since 13 July 2026.

On 3 July 2026 Dr Ma completed the Employee Capability Checklist on Metro South's own form. It certifies me fit with restrictions and records that usual switchboard operational duties remain suitable. I have remained ready and available to work within those restrictions throughout, and I remain so now.

My pay advice of 12 August 2026, for the fortnight 20 July to 2 August 2026, records me as on "NP Sick Leave" for 76 hours with gross pay of \$0.00. I did not apply for sick leave and I am not unfit for work.

2. WHAT I APPLY FOR

In the interim, and so that my entitlements can be processed, I apply as follows:

- Long service leave, on full pay, from 13 July 2026 being the date from which I have received no wages through to the current date and continuing; and
- when my long service leave entitlement is exhausted, annual leave, to commence immediately from that point and to continue until further notice.

To the extent that any absence over that period has already been recorded as unpaid sick leave, I ask that the coding be corrected and replaced with the leave applied for above.

I confirm I am not applying for personal leave, as I am not unfit for work.

3. THE AVAC

I ask that the AVAC be submitted and processed today, and that I be given the reference number once it has been done, together with confirmation of the pay run in which the payment will be made.

I ask this because of the financial position set out below, and because the cut-off for the next available pay run is imminent.

4. THIS APPLICATION IS MADE UNDER PROTEST, AND SUBJECT TO RE-CREDIT

This application is made under protest and without prejudice to my position, and on the following express basis:

- I remain ready, willing and able to work within the restrictions recorded in the Employee Capability Checklist of 3 July 2026, and I have been since 3 July 2026;
- I apply for this leave only because I have received no wages since 13 July 2026 and no payment has been made to me in the interim;
- my concerns regarding pay, the leave debited since 3 July 2026, and the adjustments recorded in the Employee Capability Checklist of 3 July 2026 remain unresolved and in dispute, and nothing in this application resolves, waives or narrows any of them; and
- I ask that the leave taken under this application be re-credited to me in the event that wages are paid, or found payable, for any part of the same period, or in the event that the recording of my absence over that period is later corrected or determined otherwise.

I make this application on the understanding that it does not prevent that outcome, and I reserve my position in full.

5. FINANCIAL HARDSHIP

I want to be plain about why I am asking for this to be done today rather than in the ordinary course.

I have had no income since 13 July 2026. As a consequence I have had to give up my accommodation. I am not raising this to make a point; I am raising it because it is the reason for the urgency, and because I would rather tell you than have it become apparent later.

If there is any mechanism available for an urgent or off-cycle payment in cases of financial hardship, I would be grateful if you could tell me and I will apply for it.

6. IF ANY PART OF THIS CANNOT BE ACTIONED

If any part of this application cannot be actioned today, could you please let me know today which part, and why, and when it will be actioned. A short note is entirely sufficient.

Thank you for your assistance.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

coryshepherd1@hotmail.com

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Cory Shepherd (388372) — Part 1: the sick-leave coding since 3 July · Part 2: payment

From: Cory Shepherd

coryshepherd1@hotmail.com

To: Chloe Taylor Chloe.Taylor3@health.qld.gov.au,
PayrollMetroSouth

PayrollMetroSouth@health.qld.gov.au

Cc: Jacqui Roberts

Jacqui.Roberts@health.qld.gov.au, Emily Petering
emily.petering@together.org.au

Sent: Thursday 3 September at 13:30

Dear Jacqui, Chloe and Payroll,

I am addressing this to you, Jacqui, because Chloe has done what she can and Payroll's answer this morning was that an ad hoc payment cannot be processed before 9 September. I have set out below why

that is not correct on Queensland Health's own policy, and I am asking you, as the senior HR contact on this matter, to see that it is resolved rather than left to run to the next reply.

Two parts. The attachment has three sections: the seventeen emails that bear on this, each reproduced once as sent, from 2 July to this morning; two tables, the leave requests and the payment position by fortnight; a copy of HR Policy C13, Payment of salaries and wages; and a copy of Directive 12/24, Special Leave. I refer to both in Part 2.

PART 1 — THE "SICK LEAVE – NO PAY" CODING

I have been asking for leave to be applied correctly since 3 July 2026, and the coding I am objecting to began before that.

Your email of 2 July said that unless I advised otherwise, shifts I had not worked would be processed as sick leave. On 3 July at 2:49 pm you said you would continue to process as "Sick leave / Sick leave no pay" because the leave type had not been confirmed (email 3). At 3:18 pm the same day I confirmed it: annual leave, not sick leave without pay (email 4). "Sick Leave – No Pay" has been applied every day since, and I have asked for it to be corrected on 10 July, 13 July, 28 July, 5 August, 24 August, 31 August and today.

"Sick Leave – No Pay" has been applied to my record from 3 July 2026, the day I was held out of the workplace — and, under your email of 2 July, to the shifts not worked before that date. It was applied against my written instruction of the same day.

This objection was made on the day the coding was first raised with me, before any question of long service leave eligibility arose, and on every occasion since. It is not a request made after the fact. I ask that the coding be corrected from the first date it was applied, and that the correction be made before my long service leave eligibility is recalculated in the audit.

PART 2 — PAYMENT

An ad hoc payment can be made, and it is the process that exists for exactly this situation.

Queensland Health's HR Policy C13, Payment of salaries and wages (QH-POL-188, 23 June 2025), section 8, describes the ad hoc payment process as one that "enables employees to request a payment that was missed or requires correction outside of the regular pay day schedule without waiting for the next "pay cycle". The certified agreement requires pay that is not correct to be corrected in a timely manner (EB12 cl 3.8.1), and the agreed mechanism for that is an ad hoc payment (EB11 cl 3.8.3). One

was processed for me on 11 March 2025 (log 4045255). The statement that a payment cannot be processed before 9 September describes the pay cycle. It is not the ad hoc process, and I do not accept it.

The pay that was missed. I was paid nothing on 26 August for the fortnight 3 to 16 August, coded "Sick Leave – No Pay" against my written instruction, and nothing on the three pay days before it. That fortnight is the "most recent prior pay period" in the policy's definition, and it is the payment I am asking for by ad hoc payment now. Process 24926366 covers 17 to 30 August only; I ask that it be confirmed in the 9 September run, but it does not answer the fortnights before it.

What the policy requires of the line manager. Section 2 provides that a line manager, when notified by an employee of an incorrect wage payment, "must take all steps to rectify the error including ... preparing and submitting further forms". I gave that notice on 3 July and on every fortnight since.

The attendance variation form submitted for 17 to 30 August is the corrective form; I ask that the same form be submitted today for 3 to 16 August, and for 3 July to 2 August, with the leave type I have instructed, and that the ad hoc payment pending under enquiry 4471091 be processed against it.

Section 8 also provides that where an employee is unable to access myHR the line manager can lodge the ad hoc request on the employee's behalf; I could not access myHR until 2 September. The request was lodged on Wednesday 2 September, within the policy's timeframe for payment before the next pay day.

Under section 9 of the same policy I also ask for a review of my pay entitlements from 3 July 2026, since my pay has been continuously incorrect from that date, to run with the leave audit Payroll has

opened.

If the ad hoc payment is refused, I ask for the reason in writing, the policy relied on, and the name of the officer who decided it. If it is not resolved locally, I will ask the union to escalate it to the payroll point of contact provided for in the certified agreement (EB12 cl 3.8.2).

Separately, and so that I am paid from here:

1. Long service leave from 13 July 2026, not 4 September. On the dates recorded in the agreement under which I was reinstated, commencement 25 March 2019, reinstatement effective 20 September 2024 with continuity preserved, seven years' continuous service fell due on 25 March 2026, and long service leave was available to me from that date. I have no other leave that could cover the period from 13 July: my recreation leave balance is 51.31 hours. I therefore apply for long service leave on full pay at 76 hours per fortnight from 13 July 2026, continuing until further notice. If Payroll maintains 4 September as the eligibility date, the difference between the two dates is the unpaid coding I have objected to since 3 July, and I ask that the audit resolve it in my favour. In the meantime, and without prejudice: (a) my recreation leave, 51.31 hours, is to be applied to 3 to 16 August so that the ad hoc payment can be made now, to be re-credited when long service leave is applied from 13 July; and (b) long service leave is to be entered from 4 September at the latest so that it is in the fortnight ending 13 September, and no further "Sick Leave – No Pay" is to be entered from that date.

2. Special leave on full pay from 3 July 2026. There is a third leave type, and it is the one that fits an absence the employer directed. Directive 12/24, Special Leave, applies to every employee covered by the certified agreement (EB12 cl 9.12). Clause 6.1 provides that a chief executive "may approve paid leave for employees for any purpose,

with duration appropriate to the purpose of the leave", beyond five days where the chief executive "considers that circumstances warrant" it. Clause 6.5 provides that in determining an application the chief executive "must consider ... the impact on the employee if the requested leave is not approved". Special leave is not debited from any leave account. I applied for it on 28 July 2026 (email 13, point 4) and again on 5 August (email 19), for each day from 3 July on which I was certified fit with restrictions, available, and directed not to attend. The reply of 29 July did not address it, the reply of 30 July did not address it, and no decision has been communicated since. I ask that the application be put to the delegate now, and that I be given the decision in writing, with reasons, including how the clause 6.5 considerations were weighed: the reason is an absence I did not choose; the duration is **62 days** and continuing; and the impact of refusal has been no income since 15 July. If special leave on full pay is approved from 3 July, the recreation leave and long service leave applied for above are to be re-credited in full.

3. Written confirmation of what will be paid on 9 September under process 24926366 — the gross amount, the hours, and the leave types debited — and whether it was processed at 76 hours or at 0.5 FTE. If at 0.5 FTE, the balance to be adjusted in the next run. The last reduced-hours arrangement ended on 28 June 2026 and nothing has replaced it.

4. The daily "Sick Leave – No Pay" entries. Section 6 of the policy permits a line manager to submit leave on an employee's behalf only where the employee's request for leave is documented in writing and the supporting documents are retained. My written request of 3 July was for annual leave. I ask for a copy of the written request and the supporting documents on which each "Sick Leave – No Pay" entry since 3 July was made.

Everything applied for is on the basis set out in my application of 31 August, paragraph 4, under protest and subject to re-credit. I remain certified fit with restrictions and available to work.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

coryshepherd1@hotmail.com

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Thursday, September 3, 2026 10:27:51 am

To: Cory Shepherd <coryshepherd1@hotmail.com>;

PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>; Emily Petering

<emily.petering@together.org.au>

Subject: RE: Cory Shepherd (388372) RE: Please respond to Chloe Taylor regarding long service leave and why it can not be actioned.

Hi Cory,

Please see the below email from Payroll.

I understand this may be frustrating, particularly in relation to your Long Service Leave. I have requested further information from Payroll regarding why your eligibility date is showing as 04/09/2026, as I appreciate you would like some clarity around this.

Unfortunately, Long Service Leave cannot be backdated & paid prior to the date you become eligible.

I have also confirmed with Payroll this morning on the phone regarding the ADHOC payment for the 17/08/2026–30/08/2026 pay cycle. Unfortunately, this cannot be processed prior to Wednesday 9th

September 2026.

I appreciate this may not be the outcome you were hoping for, and I will continue to follow up where I can, to ensure everything is clarified and processed as soon as possible.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)



Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past and present.

From: PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>

Sent: Thursday, 3 September 2026 9:05 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Subject: RE: Cory Shepherd (388372) RE: Please respond to Chloe Taylor regarding long service leave and why it can not be actioned.

Hi Chloe,

You aren't able to put in LSL leave as Cory's LSL eligibility date is only from 04/09/2026.

I can see a log has been created for a leave audit, a CSO is currently working on it however still in progress.

Kind Regards,



John Layson
Client Service Officer,
Metro South Payroll Team,

P 3608 5985

John.Layson@health.qld

E [.gov.au](mailto:_.gov.au)

Meadowbrook Service

Centre

Payroll Transactional Services,
Corporate Enterprise Solutions,
Corporate Services Division,
Queensland Health |
Queensland Health

W health.qld.gov.au

Level 1, 11-13 University

A Drive, Meadowbrook

QLD 4131

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Dear mind, remember to make time for you
Visit mentalwellbeing.initiatives.qld.gov.au



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From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Wednesday, 2 September 2026 4:16 PM

To: Cory Shepherd <coryshepherd1@hotmail.com>;

PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;

Emily Petering <emily.petering@together.org.au>

Subject: Cory Shepherd (388372) RE: Please respond to
Chloe Taylor regarding long service leave and why it can
not be actioned.

Afternoon Cory,

I am so sorry – I gave you the incorrect number – I
entered an 8 when it should have been a 2! The correct
Process number should 24926366 rather than 2498366,
I have snipped a copy of the process below.

Attendance Variation and Allowance Claim (AVAC)

Process Number: 24926366 Status: Completed

Process Details

Field	Value
Updated By	Admin Process
Item Date	24/09/2026
Process ID	24926366
Personnel Longservice Number	1410111
Person ID	1410111
Organisation	QLD Health
Organisation Code	1410111
Organisation Name	QLD Health
Organisation Address	1410111

Attachments

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In relation to Long Service Leave, please see below. You
are most welcome to follow this up with payroll
however it is still pending investigation.

Enquiry Status

Log Number: 4469554

Current Status: Pending Investigation

Lodgement Date: 31-Aug-2026

Kind regards,

Chloe

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Wednesday, 2 September 2026 2:05 PM

To: PayrollMetroSouth

<PayrollMetroSouth@health.qld.gov.au>; Chloe Taylor

<Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;

Emily Petering <emily.petering@together.org.au>

Subject: Re: Cory Shepherd (388372) RE: Please respond to
Chloe Taylor regarding long service leave and why it can
not be actioned.

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Dear Chloe,

Thank you for your correspondence confirming that
all AVAC submissions lodged on 31 August 2026
have been processed.

I have now successfully reset myHR and regained
access to my account. After reviewing the records
available to me, I have been unable to locate any
enquiry, submission, correspondence, or
documentation relating to the AVAC request
associated with PRN 249 863 66.

In addition, I contacted Payroll by telephone today
at 13:24pm and was advised that there is no
information recorded against PRN 249 863 66.
Payroll further confirmed that all AVAC
submissions lodged on 31 August 2026 had been
processed on that date.

I was also advised by Payroll that long service leave
or where a leave type cannot be processed, or

where a request is rejected, the reason is ordinarily identified during the actioning process. In light of this advice, I would be grateful if you could clarify what information or response you are currently awaiting from Payroll in relation to my long service leave application.

Given that I have been advised on two occasions—on 31 August 2026 and again on 2 September 2026—that an enquiry had been submitted, I would appreciate receiving copies of the following:

- The enquiry, submission, or request that you have confirmed was lodged.
- Any correspondence, records, or communications sent to Payroll regarding my long service leave application.
- Any documentation confirming the processing, status, or outcome of the AVAC request.
- Any correspondence relating to difficulties in processing my long service leave, including any advice received as to why the leave could not be applied.
- Any responses received from Payroll concerning my long service leave entitlement or application.
-
- This matter is of considerable importance to me. The ongoing delay in resolving my leave and pay arrangements has caused significant personal hardship, and I am seeking to establish a clear understanding of the steps that have been taken and any outstanding issues that remain.
-
- To assist in resolving the matter with yourself and payroll as quickly as possible, I would be grateful if all relevant correspondence, enquiries, and records could be provided at your earliest convenience.
-

I appreciate your assistance and look forward to your response.

-

Kind regards,

-

Cory

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Wednesday, September 2, 2026 1:00:56 pm
To: PayrollMetroSouth
<payrollmetrosouth@health.qld.gov.au>; Emily Petering
<emily.petering@together.org.au>
Subject: Fw: Cory Shepherd (388372) RE: Please respond to Chloe Taylor regarding long service leave and why it can not be actioned.

Good afternoon,

Please respond to Chloe Taylor and resolve the issue with long service leave.

Kind regards,

Cory Shepherd

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Sent: Wednesday, September 2, 2026 12:42:58 pm
To: Cory Shepherd <coryshepherd1@hotmail.com>
Cc: Jacqui Roberts
<Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au
<emily.petering@together.org.au>; Terry McQuillan
<Terry.McQuillan@health.qld.gov.au>
Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Hi Cory,

Thanks for your email.

PRN 249 863 66 has been processed for the fortnight ending 30th August 2026 and will be paid in the normal pay cycle on Wednesday 9th September 2026. I have applied S/L and A/L in lieu of S/L for this pay cycle.

Regarding your Long Service Leave from 13 July 2026, the system is currently not allowing me to apply the leave requested. I have made enquiries with Payroll and am currently awaiting their advice. I will provide an update once I receive further information.

Please see below your current leave balances as of 2nd September 2026 – Please note that leave balances shown below are prior to AVAC being processed and may therefore change once pay cycle is complete.

Cory Shepherd		By: Cory Shepherd
Leave Balances as of 02/09/2026		
Leave Type		
Leave Balance		
Leave Type		
Leave Balance		
Leave Type		
Leave Balance		
Leave Type		
Leave Balance		

If you are having trouble with access, you can call IT Support [1800 198 175](tel:1800198175) and reset your log in details.

Kind regards,
Chloe

Chloe Taylor
Switchboard Manager | Local Telecommunications Representative | Switchboard Services
Logan Hospital | Metro South | Queensland Health
P [\(07\) 2891 9190](tel:0728919190)
E chloe.taylor3@health.qld.gov.au
W www.health.qld.gov.au
A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
T [Call me on Teams](#)



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From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Wednesday, 2 September 2026 11:07 AM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au

Subject: Re: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

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safe.**

Follow up

Dear Chloe,

I am asking for an ad hoc payment to be processed today.

The leave has already been applied and the AVAC processed under PRN 249 863 66, so the entitlement exists, I am asking only that it be paid now rather than held to the next pay run.

I have received no wages since 13 July 2026. That is fifty-one days and three pay runs, and I have already had to give up my accommodation.

I should also mention that I am currently unable to log into MyHR. I cannot submit a leave application, check my balances, or see what has been applied to my record, so I am dependent on you and on Payroll for both the payment and for confirmation of what has been processed. If my access can be restored, I would be grateful and, in the meantime, could the information simply be provided to me by email.

If you cannot raise an ad hoc payment yourself, could you please tell me today who can, and I will make the request to them directly.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

coryshepherd1@hotmail.com

From: Cory Shepherd <coryshepherd1@hotmail.com>

Sent: Monday, August 31, 2026 4:06:26 pm

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: Re: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Dear Chloe,

Thank you for actioning this today and for the reference number — I appreciate the quick turnaround.

Four short points so the record is accurate.

1. Leave type. To confirm my application: I did not apply for sick leave — I am not unfit for work, and the Employee Capability Checklist of 3 July 2026 certifies me fit with restrictions. My application was for long service leave and, on its exhaustion, annual leave. Could you please confirm exactly which leave types and how many hours were applied for 17–30 August 2026 under PRN 249 863 66. If any personal or sick leave has been debited, could it please be re-credited and replaced in accordance with my application.

2. Hours. Could the leave please be applied at my substantive hours of 76 per fortnight rather than at 0.5 FTE. The reduced-hours arrangements this year were temporary changes to working conditions: the most recent, approved on 9 June 2026, had an effective period of 25 May 2026 to 28 June 2026 and was not extended.

No variation has been in force since 28 June 2026, and my pay advice of 12 August 2026 itself records my absence at 76.00 hours for the fortnight. If a different arrangement has been recorded for any period after 28 June 2026, could you please let me know what it is and the document it is based on.

3. The period from 13 July. My application was from 13 July 2026, being the date from which I have received no wages. I understand the long service leave question is with Payroll — thank you for raising it promptly. Once it is resolved, could the leave please be applied back to 13 July 2026 as an adjustment, and the "NP Sick Leave" recorded for the earlier fortnights corrected at the same time.

4. Long service leave — what may assist Payroll. My employment commenced on 25 March 2019, and my reinstatement in February 2025 took effect from 20 September 2024 with continuity of service preserved. I therefore reached seven years' continuous service on 25 March 2026 and am entitled to take long service leave under clause 22(c) of the Award. If the system is showing a later continuous service date, it may not have been corrected following my reinstatement — that would explain the error, and Payroll can verify it against my record. I have separately written to Payroll today about my service record and leave balances. Could you also confirm that the payment for 17–30 August will be processed urgently?

For completeness, everything actioned today, and any adjustments, remain on the basis set out at paragraph 4 of my email this morning — under protest and subject to re-credit.

Thanks again, Chloe.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Monday, August 31, 2026 3:34:24 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>

Cc: Jacqui Roberts

<Jacqui.Roberts@health.qld.gov.au>;

emily.petering@together.org.au

<emily.petering@together.org.au>

Subject: RE: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

Good afternoon Cory,

Thank you for your email. I have applied your S/L and A/L in lieu of S/L for your absence from the workplace through an AVAC as requested, for the past fortnight 17th August 2026 – 30th August 2026. As you are on reduced roster 0.5 FTE, I have applied the leave as per reduced shifts - PRN 249 863 66

About your long service leave from the 13th July 2026 - the system is currently not allowing me to apply LSL, so I have made enquiries with Payroll to understand why this may be and I will get back you as soon as I have an update.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Local Telecommunications Representative | Switchboard Services

Logan Hospital | Metro South | Queensland Health

P (07) 2891 9190

E chloe.taylor3@health.qld.gov.au

W www.health.qld.gov.au

A Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

T [Call me on Teams](#)

DELIVERING
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Sent: Monday, 31 August 2026 10:59 AM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Cc: Jacqui Roberts <Jacqui.Roberts@health.qld.gov.au>;
emily.petering@together.org.au

Subject: Cory Shepherd (388372) — application for long service leave and annual leave, and request for AVAC to be processed today

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Chloe,

I am writing to apply for leave and to ask that the AVAC be processed today. I have copied Jacqui Roberts.

1. MY POSITION

I have not been working since 3 July 2026. I have received no wages since 13 July 2026.

On 3 July 2026 Dr Ma completed the Employee Capability Checklist on Metro South's own form. It certifies me fit with restrictions and records that usual switchboard operational duties remain suitable. I have remained ready and available to work within those restrictions throughout, and I remain so now.

My pay advice of 12 August 2026, for the fortnight 20 July to 2 August 2026, records me as on "NP Sick Leave" for 76 hours with gross pay of \$0.00. I did not apply for sick leave and I am not unfit for work.

2. WHAT I APPLY FOR

In the interim, and so that my entitlements can be processed, I apply as follows:

- Long service leave, on full pay, from 13 July 2026 being the date from which I have received no wages through to the current date and continuing; and
- when my long service leave entitlement is exhausted, annual leave, to commence immediately from that point and to continue until further notice.

To the extent that any absence over that period has already been recorded as unpaid sick leave, I ask that the coding be corrected and replaced with the leave applied for above.

I confirm I am not applying for personal leave, as I am not unfit for work.

3. THE AVAC

I ask that the AVAC be submitted and processed today, and that I be given the reference number once it has been done, together with confirmation of the pay run in which the payment will be made.

I ask this because of the financial position set out below, and because the cut-off for the next available pay run is imminent.

4. THIS APPLICATION IS MADE UNDER PROTEST, AND SUBJECT TO RE-CREDIT

This application is made under protest and without prejudice to my position, and on the following express basis:

- I remain ready, willing and able to work within the restrictions recorded in the Employee Capability Checklist of 3 July 2026, and I have been since 3 July 2026;
- I apply for this leave only because I have received no wages since 13 July 2026 and no payment has been made to me in the interim;
- my concerns regarding pay, the leave debited since 3 July 2026, and the adjustments recorded in the Employee Capability

Checklist of 3 July 2026 remain unresolved and in dispute, and nothing in this application resolves, waives or narrows any of them; and

- I ask that the leave taken under this application be re-credited to me in the event that wages are paid, or found payable, for any part of the same period, or in the event that the recording of my absence over that period is later corrected or determined otherwise.

I make this application on the understanding that it does not prevent that outcome, and I reserve my position in full.

5. FINANCIAL HARDSHIP

I want to be plain about why I am asking for this to be done today rather than in the ordinary course.

I have had no income since 13 July 2026. As a consequence I have had to give up my accommodation. I am not raising this to make a point; I am raising it because it is the reason for the urgency, and because I would rather tell you than have it become apparent later.

If there is any mechanism available for an urgent or off-cycle payment in cases of financial hardship, I would be grateful if you could tell me and I will apply for it.

6. IF ANY PART OF THIS CANNOT BE ACTIONED

If any part of this application cannot be actioned today, could you please let me know today which part, and why, and when it will be actioned. A short note is entirely sufficient.

Thank you for your assistance.

Kind regards,

Cory Shepherd

Employee number 388372

0417 400 227

coryshepherd1@hotmail.com

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Re: Subject: Re: Cory Shepherd (388372)

— urgent food and payment today

From: Matthew Beetham from Together

Queensland representation@together.org.au

To: coryshepherd1@hotmail.com

coryshepherd1@hotmail.com

Sent: Thursday 3 September at 14:52

Hi Cory,

Thank you for your email, and I am sorry to hear about your situation.

We wanted to let you know about a service called the Workers' Psychological Support Service (WPSS), which is available to Together members. It is set up to assist workers with work-related physical or mental health issues or injuries (including a lack of support from management).

We would recommend giving them a call or sending a message via <https://wpss.org.au/> or [1800 370 732](tel:1800370732).

They provide connections to a wide range of services including mental health support, work safety issues, and financial counselling.

We have had very positive feedback from other members who have used them, but it is completely your choice as to whether you are interested in speaking with them.

Take care,

Matthew Beetham

Member Assist
Together ASU



Ph: [1800 177 244](tel:1800177244)

E: representation@together.org.au

Visit the [Member Only Portal](#) for instant access to a wealth of information on your rights at work, as well as a growing library of Frequently Asked Questions.

Together recognises and respects the Aboriginal and Torres Strait Islander peoples of the lands upon which we undertake our work to promote and defend the interests and values of members. We acknowledge the Traditional Owners and custodians who have cared for this country for thousands of years.

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On Thursday, 3 September 2026, 2:15:39 pm

+1000, Cory Shepherd

<coryshepherd1@hotmail.com> wrote:

Hi Emily,

Thank you for letting me know.

I understand you need time to review the material and I am not asking you to answer the substantive matters today.

I do need to make clear that I have not eaten properly in days and I have no funds available for food. I am asking only whether you can make a call today to Payroll, Jacqui, or the appropriate contact to see whether the ad hoc payment can be actioned urgently, or tell me who I should contact directly.

If you have any member welfare, hardship, or emergency-support option available through Together, could you please let me know today.

I am available on [0417 400 227](tel:0417400227) at any time.

Thank you,
Cory

Get [Outlook for Android](#)

From: Emily Petering from Together Queensland

<representation@together.org.au>

Sent: Thursday, September 3, 2026 2:10:44 pm

To: Cory Shepherd

<coryshepherd1@hotmail.com>

Subject: Re: Cory Shepherd (388372) —

application for long service leave and annual leave,
and request for AVAC to be processed today

Hi Cory

I was out of the office yesterday (Wednesday)
and in meetings with members this morning.

I am currently reviewing the 14 emails I've
received on your matter this week. My plan is to
review them as I am able over the next few days
and reply to you with answers to your specific
questions in the below by the end of the week.

Kind regards

Emily Petering

Member Assist
Together ASU

Ph: 1800 177 244

E: representation@together.org.au



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information on your rights at work, as well as a growing library of
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Prefer fewer emails from me? Click [here](#)

On Wednesday, 2 September 2026,
12:14:39 pm +1000, Cory Shepherd

<coryshepherd1@hotmail.com> wrote:

Hi Emily and Heath,

I need the union to act on this now, and I would like to speak with you by phone today if you can manage it.

Where I am

I have received no wages since 13 July 2026 over fifty-one days, three pay runs. I have lost my accommodation. I am certified fit with restrictions on Metro South's own form and have been ready and available since 3 July, and no instrument for keeping me out has ever been identified despite my asking on 4 August. I cannot log into MyHR, so I cannot even see what has been applied to my record.

You were copied on Monday's exchange with Chloe, so you saw it: my formal application produced the first payment action in 59 days — but for one fortnight only, at "0.5 FTE" on an arrangement that expired on 28 June, with sick leave in the mix, and with long service leave blocked "by the system". I corrected all of that within 32 minutes. Nothing has come back since Monday afternoon; not on the payment, not on the coding, not on long service leave, and not from Payroll on anything. I have today asked for an ad hoc payment to be processed immediately.

The attached two-page brief has the whole record: every request since 3 July, the key passages verbatim with timestamps, and what I am asking for.

What I am asking the union to do

1. Start a wages claim. I have been ready, willing and able within the certified restrictions since 3 July. This does not depend on the medical process and I do not want it to wait behind it any longer.

2. Get the ad hoc payment moving. The entitlement is already approved under PRN 249 863 66. An approved entitlement, unpaid, with a member who has lost his housing, is something I hope the union can move with a phone call where my emails have not.

3. Fix the coding and the rate. Sick leave removed and replaced in accordance with my application; leave applied at my substantive 76 hours, not "0.5 FTE"; and the full period from 13 July adjusted, not one fortnight. If MSH defends the 0.5 FTE, then they are paying my leave on a roster they refuse to roster me to and I would like the union to put that to them.

4. Long service leave. The system "will not allow" it. I commenced on 25 March 2019 and my February 2025 reinstatement preserved continuity, so seven years fell due on 25 March 2026. If Payroll's service date says otherwise, it was never corrected after reinstatement and has been wrong for eighteen months.

5. A decision. That MSH either return me to work within the 3 July restrictions by a stated date, or give me a written decision declining to, identifying the instrument and the officer. I would rather have a decision I can respond to than continue with none.

6. A referral to Hall Payne, for advice on a personal injury matter arising from my employment. I understand a member referral carries more weight than a cold

approach, and I would be grateful if you could make the introduction this week.

I want to be straightforward with you: this is causing me severe distress and I am not coping with it financially or otherwise.

Could you call me on [0417 400 227](tel:0417400227) today?
Any time.

Thank you, Emily.

Cory Shepherd

[0417 400 227](tel:0417400227)

coryshepherd1@hotmail.com

From: Emily Petering

<Emily.Petering@together.org.au>

Sent: Wednesday, September 2, 2026

11:07:56 am

To: Cory Shepherd

<coryshepherd1@hotmail.com>

Subject: Automatic reply: Cory Shepherd
(388372) — application for long service leave
and annual leave, and request for AVAC to be
processed today

Thank you for your email. I'm currently out of
the office, returning Thursday 3 September.

If your matter is urgent, please contact
assistance@together.org.au Otherwise, I will
respond when I'm back.

Kind regards

Emily

PAY AND THE EXCLUSION — THE RECORD

Cory Shepherd (388372), AO3, Switchboard Services, Logan Hospital · 2 July – 31 August 2026 · prepared for Together Queensland

THE POSITION IN FIVE LINES

- I have been held out of the workplace since 3 July 2026 and have received no wages since 13 July 2026.
- On 3 July 2026 Dr Ma certified me fit with restrictions on Metro South's own Employee Capability Checklist; it records that usual switchboard operational duties remain suitable. I have been ready, willing and available throughout.
- No instrument for the exclusion has ever been identified, although I asked in the Stage 1 process on 4 August 2026.
- My pay advice of 12 August 2026 records me on "NP Sick Leave" for 76.00 hours, gross \$0.00. I never applied for sick leave.
- As a consequence of the non-payment I have given up my accommodation.

EVERY WRITTEN REQUEST FOR PAYMENT — ELEVEN IN 59 DAYS

#	Date	To	The ask	The answer
1	3 Jul, 3:18 pm	Ms Taylor	"process the balance as annual leave rather than sick leave without pay"; confirm right to attend	continued processing as "Sick leave / Sick leave no pay"
2	10 Jul, 8:35 am	Ms Forrest / IM	"confirm that these shifts will be treated as paid time and will not be deducted from my leave"; return be facilitated	unanswered
3	13 Jul, 4:39 pm	Injury Mgmt	"I ask again" — paid time; if refused, the basis in writing. Interim duties offered (finance; scanning/records)	15 Jul: pay refused — "sits with your QSuper Claim Manager"; duties refused "in any capacity"
4	22 Jul	Payroll	Payroll Enquiry 4438861	"Pending Investigation" — still, 40 days on
5	28 Jul, 5:39 pm	IM, cc union	contests the coding; QSuper is "an insurance arrangement which is not a source of wages"	29–30 Jul holding replies
6	5 Aug	Stage 1 reply	pay restoration and re-credit of leave debited since 3 July	closed without either
7	12 Aug	LBH / Harrison	\$3 Wages and hardship — "no income from any source"	13 Aug reply; no pay
8	21 Aug	Conference (verbal)	pay and leave raised (Ms Roberts, Ms Petering present)	24 Aug: "speaking with Chloe this afternoon"
9	24 Aug, 11:13 am	Ms Roberts	"the appropriate leave be applied and ... an AVAC be submitted ... to facilitate the processing of my entitlements"	no AVAC followed
10	28 Aug, 2:18 pm	Ms Roberts	AVAC update sought	none
11	31 Aug, 10:59 am	Ms Taylor	formal application — LSL from 13 July, annual leave on exhaustion, AVAC today, under protest and subject to re-credit	3:34 pm: one fortnight only (17–30 Aug), PRN 249 863 66, at "0.5 FTE", leave type including sick leave; LSL blocked "by the system", referred to Payroll
12	31 Aug, 4:06 pm	Ms Taylor	correction — substantive 76 hours; no sick leave; the full period from 13 July; urgent processing	pending

The first payment action in 59 days came at request 11 — four and a half hours after a same-day deadline with the union copied — and was one fortnight, at half rate.

FOUR PAIRINGS FROM THE DOCUMENTS — VERBATIM, WITH TIMESTAMPS

1. The sick-leave coding lost its stated basis on day one.

3 July, 2:49 pm, Ms Taylor: "As you have not confirmed leave type for the past pay period — I will continue to process as Sick leave / Sick leave no pay."

3 July, 3:18 pm, Mr Shepherd: "could you please process the balance as annual leave rather than sick leave without pay."

The leave type was confirmed 29 minutes later. "NP Sick Leave" was nevertheless carried into the August payslip.

2. The stated pay basis was corrected within 46 minutes — and relied on anyway.

13 July, 3:53 pm, Injury Management: "As you currently have an active open QSuper Income Protection claim, any payments ... would be facilitated by them."

13 July, 4:39 pm, Mr Shepherd: "I no longer have an active open QSuper Income Protection claim; that claim is now closed ... There is no income protection payment that could respond to the present situation."

15 July, Ms Harrison: pay refused — "we are not responsible for your payments whilst on this claim ... sits with your QSuper Claim Manager."

3. Alternative duties were offered on day ten and refused in any capacity.

13 July, Mr Shepherd: "I am ready and willing to perform suitable interim duties of that kind [finance; scanning/records], within my certified capacity, as an alternative to remaining off work unpaid." — 15 July: interim duties refused "in any capacity".

4. The 0.5 FTE applied on 31 August has no current instrument.

The reduced-hours arrangements of 2026 were each a "Change to Working Conditions (Temporary)"; the last (approved 9 June 2026) had an effective period of 25 May to 28 June 2026 and was not extended. The exclusion began five days after it expired. The 12 August payslip itself records 76.00 hours.

WHAT I ASK THE UNION TO TAKE UP

1. PAY, as a matter in its own right: I have been ready, willing and able within the certified restrictions since 3 July. The wages question does not depend on the medical process and should not wait behind it.
2. THE CODING AND THE RATE: the absence recoded from "NP Sick Leave" in accordance with my application; leave applied at substantive hours (76/fortnight), not "0.5 FTE"; and the full period from 13 July adjusted, not one fortnight.
3. LONG SERVICE LEAVE: the system "will not allow" LSL. My employment commenced 25 March 2019 and my 2025 reinstatement preserved continuity — seven years fell due 25 March 2026. If Payroll's recorded service date says otherwise, it was not corrected on reinstatement and needs to be.
4. A DECISION: that the Health Service either return me to work within the 3 July restrictions by a stated date, or provide a written decision declining to, identifying the instrument relied on and the decision-maker.

Everything above is documented; source emails and payslips are available on request. All leave applied for on 31 August was applied for under protest and subject to re-credit, in the application itself.

Cory Shepherd · 0417 400 227 · coryshepherd1@hotmail.com · 31 August 2026